

AFRAID TO TALK? MISPERCEIVED NORMS AND INFORMAL SAFETY NETS

RONAK JAIN* AND VATSAL KHANDELWAL[‡]

ABSTRACT. Can pessimistic beliefs about social norms reduce community support among the poor? We show that misperceptions about willingness to discuss mental and financial concerns can trap communities in a low-support equilibrium. Combining a field experiment, survey experiments, and a structural model, we find that belief correction increases demand for social support, consistent with lower perceived reputational costs. However, structural estimates indicate the induced shifts are small to escape this equilibrium. Two years later, individuals reporting prior exposure to the intervention have more optimistic beliefs, but below the levels conveyed. While belief correction does not shift the equilibrium, it increases support for more intensive interventions that could sustain community interactions over time.

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1. Introduction

Individuals rely on their social networks to receive and provide support by sharing information, exchanging resources, and managing shocks.¹ However, communities may fail to provide mutual support if individuals do not *ask* for help. For instance, individuals may avoid discussing financial or mental-health concerns if they believe doing so could harm their reputation. Such beliefs can reduce the demand for support and affect the quality of risk-sharing within the community. At the same time, beliefs about others may not be accurate (Bursztyn and Yang, 2022).

This paper asks whether misperceived social norms trap communities in a low-engagement equilibrium and constrain informal safety nets even when peers are willing to help. This question is particularly relevant in settings where households face high income volatility, limited access to credit and formal insurance, and limited policy attention (Marx et al., 2013), and therefore rely on social networks to manage economic and psychological risks. Such conditions are particularly salient in urban slum settlements, which are home to roughly 860 million people worldwide (UN-Habitat, 2013). Working with such communities of informal workers in urban India, we combine multiple rounds of data collection, including a field experiment and survey experiments, with a structural model to examine the extent to which misperceived social norms may constrain engagement with the community.

Two empirical patterns motivate our study. First, despite limited formal support, most individuals do not discuss financial or mental-health concerns with peers. Moreover, 68% underestimate others’ willingness to engage, with half of them underestimating by more than 24 percentage points.² We show these misperceptions are comparable in magnitude to those documented in other settings (Bursztyn and Yang, 2022). Second, these beliefs are consequential: individuals with more pessimistic beliefs interact less and have fewer peers for support within their networks. At the same time, networks are beneficial—each additional borrowing-lending tie is associated with a three-percentage-point reduction in the likelihood of experiencing a consumption crisis, and each additional mental-health tie is associated with significantly higher self-reported happiness.

Motivated by these patterns, we design a field experiment that provides accurate information on peers’ willingness to engage around financial and mental-health issues.³ The intervention substantially increases engagement with the community: treated individuals are 15 percentage points more likely to sign up for a savings group, 16 percentage points more willing to train as listening volunteers⁴, and contribute 29% more to finance these services. They also show a higher willingness to discuss physical-health concerns. Consistent with these results, a two-week follow-up survey reveals that treated individuals report substantially higher interaction around both financial and mental-health concerns: they are 19 percentage points more likely to have

¹See, e.g., Breza (2016); Breza et al. (2019); Elliott et al. (2019) for reviews of how networks affect economic outcomes.

²Survey evidence and additional experiments show that social desirability bias is not driving the high willingness to engage that we measure around these topics.

³A prediction exercise with a separate sample from the same setting confirms this is perceived as a strong information shock, predicted to increase engagement with the network.

⁴This is similar to the services that charities like [Samaritans](#) offer in the UK and US.

reached out to any peer about financial issues and contact one additional peer, on average, relative to the control group.

We use complementary survey data and experiments to understand mechanisms. To identify which social costs discourage engagement, we use a hypothetical-choice experiment that varies reputation concerns (gossip), signaling concerns (loss of job-related information), and interaction concerns (fear of insensitive responses). In particular, we ask respondents to predict whether an advice-taking link on financial or mental-health matters would form between two randomly chosen community members, while we vary the characteristics of the hypothetical advisor. Analyzing roughly 4,740 predictions, we find that reputation and interaction costs emerge as the main barriers, whereas signaling plays a limited role. As one might expect, these costs have significantly larger effects on predicted link formation for individuals with pessimistic beliefs about the community. We also address methodological concerns by implementing a list experiment and an experiment varying the visibility of participant responses to the surveyor, ruling out social desirability and experimenter demand. We further show that alternative explanations, such as updating beliefs about the benefits of interaction, the prevalence of distress, or social pressure, are unlikely to account for our findings.

We also check whether the baseline patterns generalize to a larger, post-pandemic sample from the same setting. We find that limited dialogue and pessimistic beliefs about others’ willingness to engage continue to be widespread, suggesting that these patterns are not specific to the original sample or time-frame of the experiment. We then test whether beliefs causally affect engagement by providing respondents with information about willingness to engage in *other* similar communities.⁵ The information continues to affect behavior, but differently from the original experiment: treated individuals who are pessimistic about their own community become *less* likely to engage, while those who are more optimistic become more likely to do so. This pattern suggests that the reference group matters, as positive information about other communities potentially introduced unintended and unfavorable comparisons.

Our empirical findings establish that beliefs about others causally affect engagement, but not why those beliefs might be inaccurate in the first place, nor whether correcting them can change behavior in the long term. To understand this, we adapt and estimate a network model (Jackson and Yariv, 2007) to explain why individuals systematically underestimate others’ willingness to engage and why correcting beliefs may not permanently shift behavior.⁶ The key theoretical insight is that individuals can systematically underestimate others’ willingness to engage because they infer the norm from the behavior of their peers, who—due to the “friendship paradox” (Feld, 1991; Jackson, 2019)—are more connected and therefore less likely to engage.⁷ Intuitively, highly connected individuals face greater reputational risk when challenging norms (Young, 2015).⁸ This can trap communities in a Bayes–Nash equilibrium with persistently low dialogue. Belief correction

⁵This intervention truthfully communicated information that was previously collected from comparable communities rather than contemporaneous information from respondents’ own communities.

⁶We use “engagement” following Jackson (2019), who studies misperceptions in settings with positive peer effects.

⁷The friendship paradox states that an individual’s friends, on average, have more friends than they do.

⁸As Young (2015) notes, “*Norm entrepreneurship is a risky undertaking for the well-connected. For this reason, politicians, religious leaders, and village elders may be among the least willing to induce norm shifts even though they are the ones most capable of doing it.*”

can therefore help shift behavior in the short run, but whether these changes persist depends on the dynamics of best responses.

We structurally estimate the model using simulated method of moments combined with an equilibrium-selection criterion, leveraging the exogenous variation in beliefs generated by the RCT. The equilibrium that best fits the data is one of low engagement. The model predicts that the belief-shifting intervention generates only short-run changes in behavior and would be insufficient to shift the community to a new equilibrium. Consistent with this, we provide suggestive longer-term evidence showing that two years later, those exposed to the treatment information hold significantly more optimistic beliefs and report higher interaction intensity, higher happiness, and lower consumption volatility. However, consistent with the prediction of the structural model, their beliefs revert below the true levels conveyed during the intervention.

Using the estimated model, we assess how large alternative interventions would need to be to convert the short-run effects of belief correction into a persistent equilibrium shift. Counterfactuals show that doing so would require large changes: perceived benefits of interaction would need to increase by roughly 50% of the estimated mean (by conducting awareness sessions, for example), or individual sensitivity to social norms would need to fall by about one-third (by setting up formal job information platforms, for example, so that individuals do not worry about reputation costs when asking for financial support). Because these counterfactuals are model-dependent, we treat this as an illustrative exercise rather than literal estimates of the effects of specific policies. The exercise shows that alternative interventions would be substantially more demanding than belief correction, especially in resource-constrained settings; belief correction can raise both the demand and financial contributions for such intensive policies.

We make two contributions to the existing literature. First, we establish a causal link between beliefs about social norms and the *demand* for informal support. We show that inaccurate beliefs can make informal support systems ineffective by deterring individuals from seeking help in the first place. While prior work has shown that trust, commitment, and enforcement constraints hinder cooperation (Ambrus and Elliott, 2021; Möbius and Rosenblat, 2016; Jackson et al., 2012; Karlan et al., 2009; Fafchamps and Gubert, 2007; Ligon et al., 2002), we highlight a friction that operates even earlier: misperceptions about norms reduce the demand for interaction itself—a necessary precursor for any support to occur. Unlike rural social networks, where ties are often shaped by caste, ethnicity, or kinship,⁹ we study urban informal settlements—an understudied setting—where such structures are weaker, and beliefs play a more central role in sustaining community support.

Second, we contribute to the literature on inaccurate beliefs and information provision (e.g., Delavande (2023); Bursztyn and Yang (2022); Haaland et al. (2020); Bursztyn et al. (2020); Jackson (2019); Perkins et al. (2005, 1999)). We provide a micro-founded explanation for why misperceptions arise and lead to a low-interaction equilibrium at the community level. Our findings also complement the evidence from social psychology that individuals underestimate others’ willingness to engage (Epley and Schroeder, 2014; Epley, 2015), extending it to

⁹See, for example, Morten (2019); Banerjee et al. (2024); Munshi and Rosenzweig (2016); Banerjee et al. (2013); Munshi and Rosenzweig (2009).

interactions in real-world networks. Structural estimation allows us to quantify these dynamics, assess the limits of belief correction, and compare it to alternative policies. Consistent with recent work on signaling and stigma constraining advice-seeking (Chandrasekhar et al., 2022, 2019; Banerjee et al., 2024), we show that belief correction operates by reducing perceived reputational costs. More broadly, our results highlight that beliefs are malleable and can both increase engagement and mobilize resources for more intensive interventions in low-resource settings.

The rest of the paper proceeds as follows. Section 2 describes the context and presents the stylized facts motivating our experiment. Section 3 discusses belief elicitation and validation, and Sections 4 and 5 outline the experimental design and main results. Section 6 presents additional experiments and survey evidence on mechanisms, along with evidence from a larger-scale experiment. Section 7 introduces the model explaining why inaccurate beliefs may arise and reduce engagement, and Section 8 presents the structural estimation, the longer-term effects, and policy counterfactuals. Section 9 concludes.

2. Context

Our sample consists of informal sector workers and their family members living in slums in Delhi, India. Most individuals work in waste picking, sorting, and recycling. We partnered with an NGO, Chintan, that promotes health and safety and advocates for the rights of waste workers. Globally, nearly 860 million people live in slums (UN-Habitat, 2013) and about 15 million earn a living through recycling waste in developing countries (Medina, 2008). This context is particularly relevant given the limited policy recognition and formal assistance available to these communities, making social networks critical as safety nets (Marx et al., 2013; Chaturvedi et al., 2018; Ivaschenko et al., 2018).

Tables 2 and 3 present descriptive statistics from the baseline survey with 352 individuals across 14 locations in Delhi (Figure A.1) in 2020. Participants are, on average, 34 years old; 35% are female; and 67% are currently employed. On average, individuals take advice from about three peers in their “advice network”, defined as the number of ties with whom they discuss mental-health concerns. They interact with roughly four peers overall, including for advice-taking, borrowing, lending, and work-related interactions (their ‘overall network’). Individuals have lived in their current locations for an average of 20 years. Only 27% are migrants, and among them, just 18% frequently communicate with contacts in their origin locations. Finally, 72% report relying on their local networks for job information.

We conducted an additional survey with 791 individuals in the same setting in 2023 to ensure that the patterns are not specific to the pandemic period. Using data from both waves, we highlight three key features that motivate our intervention.

A. Financial & Psychological Distress and Lack of Formal Assistance: Individuals in our sample have very low and volatile incomes. Nearly 45% earn \$2.5 – \$5 per day, and 35% earn less than \$2 per day. Income fluctuations are substantial: as Figure A.2 shows, the difference between the highest and lowest income over the previous six months averages 50% of the mean income. Nearly three in four individuals report facing serious financial difficulties—such as inability to

afford a healthy diet, health expenditures, or children’s education—often or very often in the past six months. Formal assistance is scarce: about half lack a bank account, and among those with one, 80% struggle to access credit. We also document high levels of psychological distress. Roughly 50% report frequently feeling overwhelmed by difficulties, consistent with the elevated stress levels reported in Table 2. Formal mental-health support is rarely accessed: roughly 90% of individuals in the control group report not feeling comfortable visiting a professional.

B. Low Levels of Interactions with Social Ties: Despite low access to formal support, dialogue around mental health and financial issues with peers is limited. Figure A.3 shows that nearly half the sample rarely discusses either topic with their social contacts. Network measures reveal a similar pattern. In Figure 1, the overall network degree distribution first-order stochastically dominates the advice network degree distribution: individuals interact with more peers in general than they do for support on sensitive topics. While not all contacts are expected to provide support, the gap is quite large, consistent with network interactions being stuck at suboptimal levels. As we discuss in Section 3, these gaps are especially large among individuals with severely pessimistic beliefs about others’ willingness to engage. Similar patterns emerge for borrowing and lending links in the additional sample.

C. Networks can be beneficial: The limited use of network interactions is particularly notable given evidence that they can improve well-being. Individuals with more borrowing–lending ties report facing fewer consumption crises.¹⁰ Each additional borrowing–lending connection is associated with a significant three-percentage-point reduction in the likelihood of experiencing a consumption crisis. An extra borrowing–lending tie is associated with a five-percentage-point lower probability of reporting below-median happiness, while an additional mental-health tie is associated with a 3.5-percentage-point higher probability of being above the median.¹¹ Although these associations are not causal, they are indicative of the important role networks can play in this setting.

3. Belief Elicitation and Validation

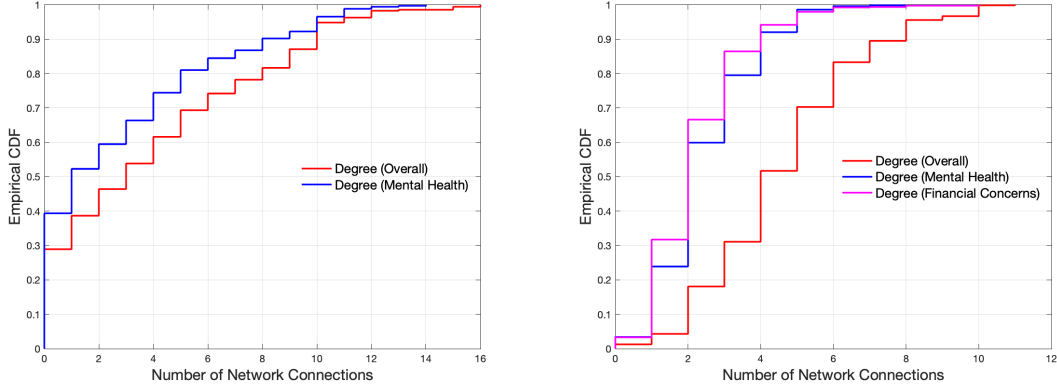
3.1. Belief Measurement. Measuring willingness to engage: To measure beliefs in a standardized way, we first clarified what we mean by physical and mental health. Specifically, before eliciting any beliefs we read out a common definition of each concept, explaining that mental-health concerns include stress, worry, or depression (along with examples of depressive symptoms) and that physical-health concerns include common body-related illnesses.¹² We then asked individuals about their willingness to engage with others on physical health, mental health, and finance/employment-related issues. ‘Willingness to engage’ was explained as the willingness to discuss ways to overcome concerns around these topics and how they may be preventing

¹⁰A consumption crisis equals 1 if the respondent reports lacking sufficient resources for basic dietary, education, or health expenditures very often in the past six months.

¹¹Similar positive correlations hold for interaction frequency.

¹²The exact wording (administered in Hindi) was: “Mental health concerns include issues such [as] stress, worry, or depression. Depression can include symptoms such as having little interest or pleasure in doing things, feeling tired or having little energy or having trouble falling or staying asleep or sleeping too much. Physical health concerns include fever, diarrhea, cough, skin problems, and other such body-related illnesses.”

FIGURE 1. Degree Distributions of Overall, Advice-Taking, and Financial Networks



Notes: The figure on the left plots the CDF of the number of connections in the overall and mental health-related advice networks in the main sample (in 2020) and the figure on the right additionally plots the CDF for borrowing-lending networks in the additional sample (in 2023). Individuals were asked to list up to ten other individuals in their community whom they interact with in any capacity, i.e., to borrow/lend, take/give advice, work with etc. (“overall networks”), whom they only take advice from regarding mental health concerns (“advice-taking network”), and whom they contact for financial concerns (“financial concerns network”).

individuals from achieving their goals. This ensures we capture meaningful dialogue rather than casual conversation.

Our measure does not assume a direction of help—whether the individual is giving or seeking support—but instead captures the underlying decision to enter a conversation at all. In practice, giving and seeking support are interdependent. Offering help matters only once others are willing to speak, and seeking help matters only if others are willing to listen. Distinguishing the two would require assuming which side must move first to break the low-dialogue equilibrium, and our design intentionally avoids this. We interpret this measure as capturing the first stage of interaction: the willingness to entertain a conversation on a topic, which governs whether individuals initiate or engage in a dialogue.

Incentivized elicitation of beliefs about others’ willingness to engage: We elicited respondents’ beliefs about their community’s willingness to engage on three topics: physical health, mental health, and financial issues—yielding 828 domain-specific belief observations. We first clarified what we mean by *community*, defining it as individuals living around the NGO center and affiliated with the NGO.¹³ For example, to measure beliefs about engagement around financial concerns, we asked how many out of any 10 community members would be willing to share and discuss financial or employment-related problems with their friends, explore ways to overcome them, and reflect on how these issues might be preventing them from achieving their goals.¹⁴ Beliefs were elicited in an incentive-compatible way. Respondents received Rs.50 (one-third of the participation incentive) if their guess was accurate within ± 1 unit on the 0–10 scale.

¹³Participants were familiar with this definition because the NGO is highly active in these areas.

¹⁴We used this format rather than percentages to ensure comprehension, based on pilot evidence.

Why is measuring beliefs about the community relevant? We intentionally measure beliefs about the community’s willingness to engage rather than beliefs about one’s immediate friends. In settings where discussing sensitive topics is not socially acceptable, such matters may remain taboo even within close friendships and therefore be unobserved (Bursztyn et al., 2020). As a result, individuals may rely on perceived community norms as the primary signal of how an interaction is likely to be received. Moreover, the costs that reduce interpersonal engagement are not confined to the dyad: fears of gossip, reputation loss, and ridicule depend on broader community perceptions, not only on the person being approached. Consistent with this, we find that community-level beliefs strongly correlate with individuals’ own engagement decisions. In a separate sample from the same setting, two-thirds of respondents predicted that engagement would increase if others learned that most community members were open to dialogue, suggesting that individuals themselves view community norms as behaviorally relevant.

Finally, as discussed in Section 6, we conduct a hypothetical link-formation experiment in which respondents predict whether one community member would feel comfortable approaching another for advice on mental and financial well-being. We experimentally vary the advisor’s characteristics. We find that community-level beliefs explain substantially more variation in link-formation predictions than advisor characteristics: over ten times more for mental health and over three times more for financial concerns.¹⁵ This suggests that even in a setting where dyadic information is available, community-level beliefs remain the more relevant predictor of whether individuals expect interactions to occur.

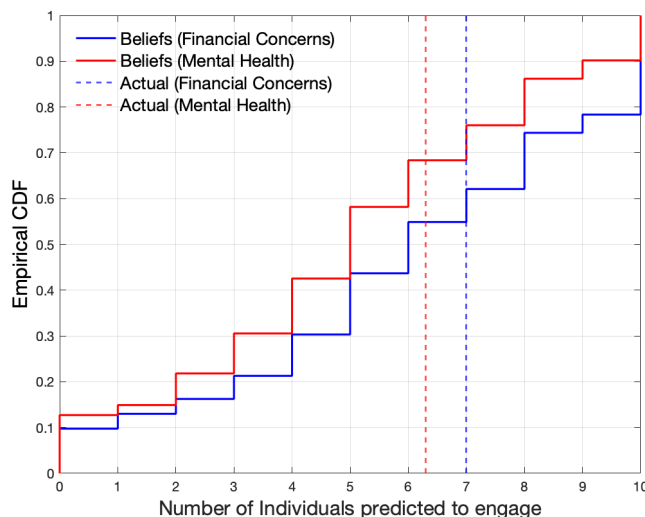
Incentivized elicitation of beliefs about stress and stigma: To shed light on mechanisms, we also elicited two additional beliefs. First, we asked individuals to predict the average level of stress in their community. Specifically, they reported how many individuals among any 10 community members would ‘often’ or ‘very often’ feel that “difficulties were piling up so high that they could not overcome them.” Comparing this prediction with the actual reports allows us to assess whether individuals overestimate or underestimate the prevalence of mental health distress. Similarly, to measure stigma, we asked respondents if they agreed with the statement: “People should stay away from individuals who have mental health issues,” and asked them how many community members would agree with the statement.

3.2. Evidence of Misperceptions. Table 3 provides summary statistics for the elicited beliefs. About 71% of individuals report being willing to engage in dialogue around financial well-being, yet they expect only about 60% of their community to do so. Similarly, 63% say they are willing to discuss mental health, but believe only about 50% of others in their community would. Figure 2 shows the distribution of these beliefs. We find substantial misperceptions about others’ willingness to engage. As shown in Figure A.5, 68% of participants underestimate the willingness to discuss mental health in their own community, and 58% underestimate engagement around

¹⁵In an OLS regression of link-formation predictions on community-level beliefs about engagement with mental well-being alone, $R^2 = 0.010$. With experimentally randomized advisor characteristics alone, $R^2 = 0.001$. The full specification with interactions gives $R^2 = 0.015$. While R^2 values are modest in absolute terms, the relative comparison is informative: community-level beliefs explain an order of magnitude more variation than the experimentally varied advisor characteristics. Community-level beliefs remain significant throughout.

financial concerns. In contrast, most individuals overestimate the prevalence of stress in the community.

FIGURE 2. Beliefs about Community Engagement



Notes: The solid lines in the figure plot the CDF of how many community members out of a randomly chosen 10 individuals believe will be willing to engage around financial concerns and mental health issues respectively. The dotted lines indicate the actual sample willingness to engage.

Figure 6 plots the distribution of the belief wedge (truth minus own belief, in percentage points) for mental health and financial well-being.¹⁶ We find that the magnitude of underestimation around willingness to discuss mental-health-related concerns is large. Among the 68% of the sample who underestimate, the mean misperception is approximately 30 percentage points and one in four underestimators has a misperception larger than 40 percentage points. Note that such misperceptions can meaningfully raise the perceived social costs of engaging on sensitive topics because individuals tend to fear worst-case reactions—such as rejection or insensitivity—which depend on perceived norms. Perceived costs can respond nonlinearly to beliefs, and misperceptions can produce even larger behavioral differences. Figure A.6 shows the same errors measured against each person’s own community rather than the whole sample, and the pattern looks very similar.

Comparison with the Literature on Misperceptions. These misperceptions are comparable in magnitude to those documented in other settings. Bursztyn and Yang (2022) review 81 studies that elicit beliefs about others and construct two summary measures. The first is the share of respondents whose beliefs are within half a standard deviation of the truth. By this measure, 32% of respondents in our data hold accurate beliefs about others’ willingness to engage with mental

¹⁶We also compare the empirical distribution of the wedge to a random-guessing null in which each individual draws a belief independently and uniformly from integers 0 to 10 (the same scale used in the survey). Across 1,000 Monte Carlo simulations, a two-sample Kolmogorov–Smirnov test rejects equality of distributions at the 10% level in 95% of draws for financial well-being (median $p = 0.006$) and 17% of draws for mental health (median $p = 0.25$).

health.¹⁷ Compared to the studies reviewed by Bursztyn and Yang (2022), our share of respondents with accurate beliefs is lower than about 60–65% of the studies. The second measure is the ratio of underestimation to overestimation. Respondents are 2.3 times as likely to underestimate as to overestimate their community’s willingness to engage with mental health. About 45% of the studies reviewed in Bursztyn and Yang (2022) report a ratio that is smaller than ours.

Social Desirability: One concern with measurement is that the high willingness to engage could be driven by social desirability bias. We do not expect this to be the case. First, around 49% of the participants report agreeing that one must stay away from those with mental health issues, indicating that respondents felt comfortable expressing potentially stigmatizing views.¹⁸ Second, participants do not uniformly express willingness to engage across topics: for example, 42% of those unwilling to engage on mental-health concerns are nevertheless willing to engage on financial matters. Third, as we show in the next section, willingness to engage is systematically correlated with baseline beliefs about peers’ willingness to engage. Taken together, these patterns suggest that respondents answered thoughtfully and that these measures are not driven by cheap talk. An additional list experiment as well as an enumerator visibility experiment discussed in Section 6.2.1 further resolve this concern by showing that willingness is not affected by participants’ ability to mask their responses from the enumerator.

3.3. Validation of Elicited Beliefs. To assess the accuracy of elicited beliefs, we correlate them with individuals’ past behavior, network characteristics, and economic outcomes, following Delavande et al. (2011). This allows us to confirm that the misperceptions we observe are not driven by measurement error. Across all checks, the correlations have the expected signs and support the validity of our measures.

Correlations of Beliefs and Individual Willingness to Engage: Participants’ beliefs about their community’s willingness to engage on mental, financial, and physical health are positively correlated with each other (Table A.1). This implies that optimism about the openness of the community on one dimension implies optimism around other dimensions as well. These beliefs are also correlated with an individual’s own willingness to engage, suggesting that they capture meaningful variation in perceived norms and behavior.

Correlations of Beliefs and Frequency of Dialogue: Beliefs are strongly associated with the frequency of actual conversations. Regressing conversation frequency on beliefs about engagement shows that more optimistic beliefs predict higher dialogue around both mental health and financial issues (Table A.2). Panel B further shows that individuals who underestimate community engagement are significantly less willing to engage in dialogue around mental health themselves.

Correlations of Beliefs and Network Gaps: We also find that pessimistic beliefs are associated with structural features of individuals’ social networks. Individuals with lower

¹⁷We classify beliefs as accurate when the absolute difference between a respondent’s belief and the relevant community-level mean is no greater than one-half of the sample standard deviation of elicited beliefs.

¹⁸Discussions with NGO staff and a psychologist suggest that the phrasing may have led participants to think of severe mental-health disorders.

perceived community openness have larger gaps between their overall and advice networks (Table A.2). These gaps—measured as the proportional difference between overall network degree and advice network degree—are especially large among “severe underestimators” (bottom 25th percentile of belief accuracy).

Correlations of Beliefs with Consumption Outcomes: Using data from the additional sample, we find that more pessimistic beliefs are correlated with higher consumption volatility and a greater likelihood of experiencing a consumption crisis (Table A.4). This highlights the behavioral and economic relevance of misperceptions.

Large-scale validation in 2023: To ensure that observed misperceptions are not specific to the COVID-19 period, we elicited beliefs in a larger sample with 791 individuals in 2023. Underestimation of community engagement remains high, and willingness to engage is similarly high across topics. As before, more optimistic beliefs are associated with having more mental-health and financial network ties (Table A.4). Full results are presented in Appendix Section F.

4. Experiment

4.1. Timeline and Design. The main experiment and endline surveys were conducted with 180 individuals from February to April 2021, with roughly half in the treatment group and half in the control group. In 2023, we implemented a larger-scale experiment with approximately 800 additional participants.¹⁹

The treatment group received two pieces of information at two different points during the survey: (i) the true average sample willingness to engage in dialogue about mental health, and (ii) information that the sample’s willingness to engage in a dialogue about financial well-being was high.²⁰ More specifically, the statements used were:

Information 1: *“Just like we surveyed you, we also surveyed other people and we have found from their responses that 6 out of any 10 individuals in your community and communities similar to yours (affiliated with the NGO) are willing to discuss their mental health concerns with their peers, how they can be preventing them from achieving their goals, and how they can be overcome.”*

Information 2: *“Just like we surveyed you, we also surveyed other people and we have found from their responses that the proportion of individuals willing to discuss their financial/work-related concerns with their peers, how they can be preventing them from achieving their goals, and how they can be overcome is high.”*²¹

¹⁹Further details are provided in Section F of the appendix.

²⁰We provided information about sample-level averages computed using data on willingness to engage from our initial surveys of about 350 individuals rather than community-level averages. We did this because some communities had low response rates, and we did not wish to shift beliefs about their community based on information obtained from small samples. Importantly, since the communities are very similar to each other, information from the entire sample is informative and helpful.

²¹Note that we do not provide a precise estimate here (the precise estimate of the proportion of individuals willing to share these concerns is 70%) because unlike mental health, where there is a higher proportion of under-estimators, in this case, 58% underestimate, and we do not wish to make them more pessimistic.

The second piece of information was provided closer to the end of the survey, before the questions related to financial outcomes. All other components of the survey were identical for the control and treatment groups.

Out-of-sample predictions about the intervention’s effects: We expect the treatment to function as a meaningful information shock, given limited dialogue around these topics and the stigma associated with discussing mental well-being. To assess whether individuals themselves viewed the information as relevant for behavior, we asked participants in our additional sample to predict how others would respond if they received Information 1. A large majority—67%—anticipated that engagement with savings groups would increase, and 42% expected this increase to be large.

4.2. Balance Tests. We show that the sample is balanced across control and treatment groups across a host of baseline variables including demographic information, network connections, dialogue intensity, beliefs about mental health, physical health, financial concerns, participant well-being, and own willingness to engage. In particular, we test balance on 44 variables and find that the sample is unbalanced only for 2 variables. These results are presented in Tables B.1 and B.2. We also regress treatment status on these baseline covariates and find that the F statistic is 0.68 and the corresponding p -value is 0.89. This suggests that the baseline characteristics are balanced across treatment and control. However, for robustness, we also run a specification that controls for any unbalanced covariates.

4.3. Specification. We regress each outcome on the treatment indicator for the individual, with robust standard errors. We additionally report p -values of wild bootstrapped t -statistics in line with Cameron et al. (2008) where we cluster the standard errors at the level of the NGO center. Further, we also conduct correction for tests of multiple hypotheses as per Benjamini et al. (2006); Anderson (2008). The outcome families used for the q -value computations follow our pre-analysis plan. We also run an additional robustness exercise where we take a very conservative approach and treat all the outcomes as one family before computing the q -values.

Finally, we also account for the fact that participants may not have always completed the endline survey as a result of which some outcomes may have received more responses than others. To address any balance-related concerns that may arise due to this, we adopt the following additional strategy as a robustness check in the appendix. We run balance tests for *each* sub-sample for which the outcome variable is non-missing. Then, we include the unbalanced controls (at 5% significance) in a conservative, robust specification. All the main results are robust to this alternative specification unless specified otherwise.

5. Results

We now present the effects of the intervention on pre-registered outcomes measured immediately after the treatment.²² These include (1) willingness to engage with the community on mental health and financial issues, (2) demand for formal support, (3) willingness to talk about other

²²The experiment was pre-registered at the AEA RCT registry: <https://doi.org/10.1257/rct.6980-2.1>. Differences between the analysis plan and the analyses reported in the paper are discussed in Appendix Section F

health-related issues, (4) demand for additional information, and (5) financial self-efficacy and stigma. Most outcomes were measured after Information 1; we indicate which outcomes were collected after both information statements. Appendix Section C.1 provides further details on how the outcomes were measured and the hypotheses we test.

5.1. Community Engagement. We expect belief correction to increase willingness to engage with the community by lowering the perceived social cost of doing so. Community engagement is defined as the mean of three binary variables capturing willingness to have useful interactions with the community: (a) willingness to train as a “listening volunteer” for the community²³, (b) willingness to contribute financially to establish this listening service, and (c) willingness to participate in an NGO-facilitated savings group to pool savings to help each other in difficult times.²⁴ The first two outcomes were collected after Information 1; the third was collected after both information statements.

As Table 4 shows, the intervention increases community engagement by 14 percentage points.²⁵ Figure 3 plots the treatment effects separately. Treated individuals are 16 percentage points more likely to enlist as volunteers (significant at 5%) and 12 percentage points more likely to financially contribute to the training. Their average contribution increases by Rs. 6.6, which is a 29% increase compared to the control group (significant at 5%).²⁶ We also find that while 67% of the control group is willing to participate in savings groups, the treatment increases this further by about 15 percentage points (significant at 5%).

These results indicate that correcting misperceptions about peers meaningfully increases individuals’ willingness to interact with their networks—both through volunteering time and financially contributing to informal support avenues. All effects remain robust to the inclusion of unbalanced controls.

5.2. Demand for Formal Support. When people learn that their peers are supportive, asking for help can feel more acceptable, which could increase formal help-seeking. At the same time, community support could substitute for formal support and lower it. Panel B of Table 4 tests this. We find that treated participants report lower hesitation to speak with a doctor about mental health concerns, which is consistent with a reduction in perceived stigma or perceived costs of violating the social norm. However, treatment also reduces participation in experimenter-run depression scoring (PHQ-9) and reduces willingness to listen to government helpline information, both measured after both information statements. These patterns suggest that individuals may consider formal and informal support as substitutes: greater comfort with community-based dialogue may reduce interest in some externally provided resources. All effects remain robust to adding unbalanced controls.

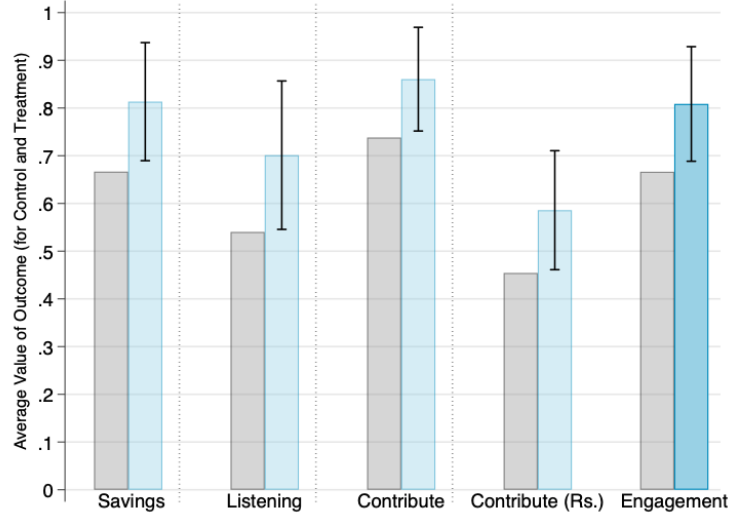
²³This captures training to become a “mental health buddy” who listens to the worries of other community members, similar to the services provided by charities like Samaritans.

²⁴The exact survey wording of these outcomes is reported in Appendix Section C.1.

²⁵All q -values are also significant at 5%.

²⁶Participants were informed that their contributions would be deducted from a Rs 50 prize for those who won based on their accuracy of guesses. Baseline beliefs were balanced, so selective pessimism about winning is unlikely.

FIGURE 3. Treatment Effects on Engagement with the Network



Notes: The above figure shows the average value of each outcome for the control group (in gray) and the treatment group (in blue) with 95% confidence intervals for the difference between the two values. “Savings”, “Listening”, and “Contribute” refer to binary variables indicating whether the individual is willing to participate in savings groups, train and volunteer for a listening service, and actually contribute to set it up. “Engagement” is an average of these three variables. “Contribute (Rs.)” is the actual monetary contribution made by the individuals, normalized to be between 0 and 1.

5.3. Other Types of Dialogue. Support from peers on one dimension of dialogue can spill over to other types of dialogue as well. Panel C of Table 4 provides evidence for sizable positive spillover effects: treated individuals are 21 percentage points more likely to report willingness to discuss physical health-related issues with peers. Given baseline stigma around discussing physical health, this suggests that shifting beliefs about norms in one domain (mental health) can generalize to other sensitive domains. The result is robust to unbalanced controls.

5.4. Demand for Additional Information. Lower perceived costs of engaging should also raise demand for information around these topics. Demand for additional information includes willingness to attend a one-hour NGO-led mental-health information session and willingness to listen to guidance on initiating sensitive conversations. The latter outcome was measured after participants received both information statements.

Table C.1 shows that the treatment does not affect participants’ demand for additional information about mental health. Admittedly, our ability to detect effects on the take-up of the information session is limited due to the high baseline take-up of the information session as 67% of the control group agrees to participate. Anecdotally, we learned that respondents perceived the session as similar to routine NGO trainings rather than as a stigmatized activity. We also find no effect on demand for guidance on having sensitive conversations.

5.5. Other Outcomes. We do not detect treatment effects on financial self-efficacy or stigma (Supplementary Appendix Table C.1). Financial self-efficacy measures confidence in (a) managing financial or employment-related concerns in the coming months, (b) financing children’s education

expenses, and (c) starting a small business. If individuals can rely on their network, self-efficacy should rise. However, we do not find any effect on any of these outcomes. While one might expect belief correction about peers’ willingness to engage on both financial and mental-health issues to affect these outcomes, the null results are consistent with the lack of existing financial risk-sharing: optimism about dialogue need not immediately translate into confidence about borrowing or financial support if such links are absent.

Finally, since the treatment corrects beliefs about others’ willingness to engage with mental well-being, we can expect a reduction in stigma around such topics. Stigma is measured using: (i) willingness to have one’s name publicly shared as a participant in the information session, (ii) a list-experiment measure of agreement with stigmatizing statements, and (iii) willingness to disclose depression scores to the surveyor (conditional on participating in the depression scoring). We do not find any treatment effects on these outcomes. Notably, 92% of control participants were willing to have their names shared, suggesting this was a weak measure of stigma—likely because NGO-run information sessions are common and not perceived as stigmatizing.

5.6. Two-week Follow-Up. We conducted a follow-up survey two weeks after the intervention in 2021. As described in Appendix Section E.1, we pre-specified all follow-up outcomes. Due to the peak of the COVID-19 crisis in India during April 2021, we were able to reach only 112 respondents (57 treated). Despite the smaller sample, balance checks on 44 baseline variables show imbalance on only 2, and robustness checks controlling for these variables confirm that attrition is not correlated with treatment status or baseline characteristics.

Table 5 shows that treated individuals are 11–19 percentage points more likely to have reached out (via phone or SMS) to at least one peer to discuss their own or others’ mental-health or financial concerns in the two weeks following treatment. Treated respondents also contacted roughly one additional peer to discuss both mental-health issues ($p < 0.05$) and employment-related concerns ($p < 0.01$). The effects are stronger for initiating contact than for receiving it, consistent with the fact that only treated individuals received the information. However, as this was the peak of the Delta wave in Delhi (an aggregate correlated shock affecting all community members at once), we do find that treated individuals report lower happiness and lower life satisfaction (Table E.1). Additional results on well-being and dictator games, as well as a discussion of potential COVID-related concerns, are presented in Appendix Section E.

5.7. Heterogeneity. We examine heterogeneity in treatment effects by baseline willingness to engage, dialogue intensity, network size, stigma, and beliefs.²⁷ The results are shown in Tables C.2-C.7 in the appendix. Below, we summarize the main patterns.

5.7.1. Heterogeneity by Baseline Engagement. Table C.2 shows that the treatment effects on community engagement are substantially larger among individuals who were initially unwilling to engage on mental-health concerns. For these respondents, the intervention increases willingness to (a) engage with their community, (b) attend a mental-health information session, and (c) listen to

²⁷Heterogeneity by baseline beliefs about stress is discussed with mechanisms in Section 6.

good practices for sensitive conversations.²⁸ Similarly, Table C.3 shows that effects are significantly larger for individuals with below-median baseline dialogue on mental-health and financial issues. This pattern is consistent with the intervention benefiting those who were least engaged at baseline, thereby helping those who need it most.

5.7.2. Heterogeneity by Baseline Networks. Table C.4 shows strong heterogeneity by the overall network size. More connected individuals exhibit larger treatment effects: each additional peer raises the probability of participating in a savings group by roughly 3 p.p., while treatment effects for individuals with zero peers are effectively zero. More connected individuals also contribute significantly more to establishing the listening service in response to treatment, reversing the baseline pattern in which they are less likely to contribute. We return to this result in Section 7, where the theoretical framework predicts larger gains for highly connected individuals.

5.7.3. Heterogeneity by Baseline Stigma. As shown in Table C.5, individuals expressing mental-health-related stigma (those agreeing that they should “stay away” from people with mental-health issues) donate Rs. 7 less to the listening service than those without stigma. However, among individuals who express stigma, treated participants donate Rs. 9 more than their control counterparts. While information on community openness does not directly reduce stigma, it substantially increases engagement among those who initially hold stigmatizing views—potentially because pessimistic beliefs about norms and expressed stigma are correlated, and the intervention relaxes perceived social costs of engagement.

5.7.4. Heterogeneity by Baseline Beliefs. Finally, Table C.6 examines heterogeneity by whether individuals underestimated community engagement around mental-health and financial issues. Our ability to detect effects is limited, but effect sizes are generally larger for underestimators, consistent with the mechanism that belief correction matters most for those holding pessimistic priors.

6. Mechanisms

6.1. Main Mechanism: Lower Perceived Costs of Violating the Social Norm. To understand the mechanisms underlying the treatment effects, we use a survey experiment with our additional sample of around 800 individuals. Each individual was asked to predict whether a link would form between two hypothetical community members, where agent *A* is a potential advisor and agent *B* needs support for financial or mental health-related concerns. We exogenously vary three characteristics of advisor *A*: (1) Network centrality (testing reputational costs—concerns about gossip), (2) Contacts in private jobs (testing signaling costs—fear of negative inferences affecting job referrals), and (3) Sensitivity training attendance (testing interaction costs—fear of being mocked or treated insensitively). Each respondent evaluated three randomly generated profiles, yielding 4,740 total predictions across mental-health and financial scenarios.

²⁸For the last measure, the interaction term is marginal, but the bootstrapped p-value is significant at 5%.

Overall, 23% of mental-health links and 24% of financial links were predicted not to form—reflecting meaningful perceived barriers even in a hypothetical setting. Table 8 shows that individuals with more pessimistic beliefs about community engagement are significantly more likely to predict that links will not form, especially when A is highly central or has attended sensitivity training. This pattern is consistent across both OLS and Probit specifications.

To understand why, we asked respondents to explain their answers. Among those rejecting links when A is central, 42% cited concerns that A would not have time, and 39% cited fear of gossip. Among those rejecting links when A had job contacts, 30% feared being judged as an unsuitable candidate for referrals—consistent with signaling concerns, even though this characteristic did not causally reduce predicted link formation on average. Surprisingly, sensitivity training attendance acted as a negative signal: respondents interpreted participation as suggesting that “something must be wrong” with A, reinforcing interaction-related concerns.

Taken together, these results indicate that reputational and interaction costs—both shaped by perceived social norms—are key barriers to forming supportive ties. Importantly, these costs are substantially higher among those with more pessimistic beliefs about others’ willingness to engage, precisely the group for whom the belief correction treatment had the largest effects. This interpretation is reinforced by qualitative interviews: participants reported fear of being shamed, mocked, or gossiped about if their request for help was rejected, describing themselves as “ashamed,” “worried they will make fun later,” and “suffocated or trapped because others can make fun of their poverty.”

6.2. Alternative Mechanisms. We next use additional experiments and survey evidence to discuss alternative explanations for the treatment effects.

6.2.1. Social Desirability and Experimenter Demand. We assess whether our findings could reflect social desirability or experimenter-demand effects using (a) a list experiment and (b) random variation in whether key responses were visible to the enumerator.

List experiment: Following standard practice (Stantcheva, 2023; Bursztyn et al., 2020), we use a list experiment for measuring sensitive attitudes while preserving anonymity. Individuals are randomly assigned to a control group or to one of two treatment groups that received either one or two statements about mental health-related and financial well-being-related engagement with peers, respectively.²⁹ Respondents only report how many statements they agreed with, masking agreement with any specific item. If social desirability drove reported willingness to engage, we would expect no difference across groups. Instead, agreement rises systematically with the inclusion of engagement-related statements (2.21 in control; 2.97 and 3.71 in treatment groups), and the differences are statistically significant. This increase is not mechanical: since respondents report

²⁹The statements are as follows: (1) Only individuals who have received formal education should enter into politics. (2) The Delhi government is taking the required steps to deal with air pollution. (3) Teachers should be paid more remuneration than film actors. (4) Individuals should take time to listen to the mental health concerns of their peers (Only to Treatment Groups 1 and 2) (5) Individuals should take time to listen to the employment or money-related concerns of their peers. (Only to Treatment Group 2).

only a total count, a higher average is informative only if they genuinely agree with the added items.

Enumerator-visibility experiment: In a separate exercise, we randomized whether willingness to join a savings group or train as a listener was reported privately or aloud to the enumerator. Responses are indistinguishable across the two conditions and do not interact with treatment status (Table D.2), indicating that neither social desirability nor experimenter demand explains the results.

Additional evidence from costly outcomes: The treatment raises costly behaviors and incentivized outcomes—such as financial contributions—which are unlikely to be driven by cheap talk. Moreover, the largest effects occur among individuals with low baseline engagement and low baseline dialogue (Tables C.2–C.3), which is inconsistent with a simple desirability story.

6.2.2. Updating Beliefs about the Incidence of Stress. The treatment could also affect behavior if individuals revise beliefs about how stressed or burdened their peers are. However, baseline data show that over 60% overestimate community stress levels, making it unlikely that lack of awareness of stress is a key constraint. Consistent with this, treatment effects do not vary by whether respondents over- or underestimate stress (Table C.7). This suggests that changes in perceived incidence of stress are not the main pathway.

6.2.3. Altruism or Social Pressure. Could individuals increase engagement out of altruism or a desire to appear supportive? Several patterns suggest this is unlikely. First, altruism alone is difficult to reconcile with the low levels of dialogue at baseline, despite widespread awareness of high stress in the community. Second, contributions to the listening service were explicitly private, and willingness to have one’s name shared publicly does not differ by treatment status. Third, the treatment *reduces* the take-up of depression-scoring and helpline information, consistent with substitution toward community-based support following a reduction in the perceived costs of interactions. These patterns are difficult to reconcile with social pressure or a desire to appear supportive, as neither visibility nor reputational incentives drive the observed behavior.

6.2.4. Updating Beliefs about Benefits of Interacting. Another possibility is that individuals update beliefs about the benefits of interacting rather than the costs. However, baseline data show that willingness to engage is already high, even among individuals who rarely engage, suggesting that perceived benefits are not the binding constraint. Moreover, while the frequency of dialogue is low at baseline, such interactions are not absent altogether; respondents could observe any existing engagement within their community—for instance, borrowing–lending ties—so it seems less plausible that they are unaware of the potential benefits. Qualitative interviews also confirm that respondents view such interactions as valuable. These patterns point to perceived social costs—not a lack of perceived benefits—as the more plausible barrier.

6.3. Large-Scale Survey and Experiment. We ran a second round of data collection and a field experiment in 2023 with roughly 800 individuals across two NGO centers for two reasons. First, we test and establish that the patterns of low dialogue and underestimation of peer willingness to engage are not specific to the pandemic. We find that the same baseline patterns of low dialogue

and pessimistic beliefs also hold in this larger sample. Roughly 30% still rarely or never discuss mental-health or financial concerns, and an even larger share underestimate their community’s willingness to engage around these topics.

Second, we test whether beliefs continue to causally affect engagement in this larger sample. We implemented a variation of the intervention in which participants received information about the willingness to engage in similar communities, without explicitly including their own.³⁰ We find that this information continues to have a significant causal impact on willingness to engage. However, the mechanisms differ from the main experiment. Individuals who are pessimistic about their own community become *less* likely to join a savings group when told that *other* communities are more willing to engage, whereas those who are optimistic about their own community respond in the opposite direction. A similar pattern arises for volunteering in listening services. Treated participants continue to make higher financial contributions to support informal interaction avenues, particularly among those who are pessimistic about the social norm.

These results suggest that the reference group may affect how individuals respond to information: information about other communities may induce relative comparisons and discourage engagement among those who are pessimistic about their own community. A detailed discussion of these results appears in Appendix Section F.

7. Theoretical Framework

The experiment establishes that beliefs about others causally affect engagement, but not why these beliefs may be inaccurate in the first place, nor whether correcting them would change behavior meaningfully in the long run. To understand this, we adapt the frameworks in Jackson and Yariv (2007) and Jackson (2019) to show how individuals in a fixed network form beliefs about others’ willingness to engage and how these beliefs affect their own decisions to interact around sensitive topics. The model illustrates how misperceptions naturally arise and motivates our information intervention.

Let N denote the set of individuals connected in an undirected network G where $g_{ij} = 1$ if i and j are linked. Individual i has degree $d_i = \sum_j g_{ij}$, and $P(d)$ denotes the degree distribution. Each person chooses whether to engage, $e_i \in \{0, 1\}$, interpreted as taking a visible action to interact around mental-health or financial concerns (e.g., participating in a savings group or training as a listening volunteer).³¹

Choosing $e_i = 1$ yields a private benefit $b_i > 0$, drawn from an atomless distribution H . Engagement involves a cost $c(d_i, 1 - E[e_{j:j \in N}])$ that increases with both the expected share of disengagement in

³⁰We informed participants: “Just as we surveyed you, about two years ago we also surveyed about 300 other people who live in a community like yours and are affiliated with the NGO. Among any 10 of them, about 6 were willing to discuss mental-health concerns with their friends. This information is about two years old, but assume it were true today.” The intervention truthfully communicated previously collected information from comparable communities (during the original experiment) rather than contemporaneous information from respondents’ own communities.

³¹We treat willingness to engage and engagement as the same object in the framework: a visible action (e.g., joining a savings group) is taken precisely by those willing to engage, so the share of individuals willing to engage coincides with the share that actually engages. Throughout, the norm is represented by $E[e_j]$, i.e., the share of the community that engages.

society $(1 - E[e_{j:j \in N}])$ and the individual's degree (d_i) . We assume that c is weakly increasing in both arguments, $\frac{\partial^2 c}{\partial d_i \partial (1 - E[e_{j:j \in N}])} > 0$, and $c(\cdot, 0) = 0$.

An individual i 's utility can be written as:

$$U(e_i, d_i; e_{j:j \in N_i}) = [b_i - c(d_i, 1 - E[e_{j:j \in N}])]e_i \quad (1)$$

Individual i engages if $b_i > c(d_i, 1 - E[e_{j:j \in N}])$, so the engagement probability is $1 - H(c(d_i, 1 - E[e_{j:j \in N}]))$. Agents know their degree, $P(d)$, and H .

7.1. Empirical Evidence for Model Assumptions. A key assumption is that engagement is more costly for more connected individuals when the perceived norm is non-engagement.

We find direct support for this assumption in our own experimental evidence. In the network prediction experiment, 42% of respondents believe that advice-taking links will not form when the advisor is highly connected because “the advisor will not have time.” Consistent with the same assumption, more connected individuals respond more strongly to the belief-correction intervention, consistent with a higher marginal cost of engaging when norms are perceived as unfavorable.³² Regressing the decision to participate in savings groups and listening services on the interaction of degree centrality and beliefs about non-engagement yields negative coefficients, suggesting that pessimistic beliefs reduce engagement particularly among the better connected.³³ We find further evidence in descriptive patterns: individuals with more connections are less likely to want to participate in savings groups (Figure G.1).

Stronger and more general support comes from intuitive mechanisms noted in existing literature. First, having more connections implies having a larger “audience,” raising the reputational risks of sharing sensitive information since more peers can gossip, judge, or update their beliefs negatively (as noted in Section 6). Second, highly connected individuals face stronger pressures to conform to existing norms and have more relationships at stake. Third, people with many connections often have a combination of strong and weak ties, increasing the probability of encountering someone who might be judgmental or insensitive. Fourth, they may simply have less time and attention to devote to such interactions. This aligns with evidence that challenging prevailing norms is especially costly for the well-connected, as Young (2015) notes:

“Norm entrepreneurship is a risky undertaking for the well-connected. For this reason, politicians, religious leaders, and village elders may be among the least willing to induce norm shifts even though they are the ones most capable of doing it.”

7.2. Equilibrium. Following Jackson and Yariv (2007), a symmetric Bayesian Nash equilibrium satisfies:

³²This is shown in Table C.4, where the interaction between degree and treatment is positive for the main engagement outcomes and significant for two of the six measures.

³³The coefficient is also negative when we either use beliefs about others' willingness to engage around financial well-being as a measure of beliefs or when we use savings groups as the measure of engagement.

$$\hat{e} = \sum_d P(d)(1 - H(c(d, 1 - \hat{e}))). \quad (2)$$

We then consider the case, following Jackson (2019), in which individuals naively estimate the social norm using only the behavior of their neighbors. Because of the friendship paradox (Feld, 1991), the degree distribution of neighbors is: $\tilde{P}(d) = \frac{d}{E[d]} P(d)$, which first-order stochastically dominates $P(d)$. Individuals, therefore, infer the norm from a more connected subpopulation, which—because c increases in degree—leads them to overestimate non-engagement.

The friendship paradox is one of several mechanisms that can generate misperceptions. We focus on it because it provides a parsimonious explanation for how biased beliefs about engagement can arise in networks (Jackson, 2019). We emphasize that the friendship paradox predicts only the *average* direction of the bias, not every individual belief. While some individuals may overestimate others' willingness to engage, the model predicts that, on average, individuals underestimate engagement.

The perceived (peer-based) equilibrium $\tilde{e}$ satisfies:

$$\tilde{e} = \sum_d \tilde{P}(d)(1 - H(c(d, 1 - \tilde{e}))) = 1 - \sum_d \tilde{P}(d)(H(c(d, 1 - \tilde{e}))) \quad (3)$$

where $\tilde{e}$ denotes the equilibrium engagement when individuals use $\tilde{P}(d)$ instead of $P(d)$. Rewriting the equation succinctly, denoting the probability that a randomly chosen peer does not engage by a , and $\tilde{a} + \tilde{e} = 1$:

$$\tilde{e} = 1 - \sum_d \tilde{P}(d)(H(c(d, 1 - \tilde{e}))) = 1 - \tilde{a} \implies \tilde{a} = \sum_d \tilde{P}(d)(H(c(d, \tilde{a})))$$

Lemma 1 implies that individuals always perceive higher non-engagement when relying on peers to make inferences.³⁴

Lemma 1. $E[H(c(d, a))] \leq \tilde{E}[H(c(d, a))] \forall a \in [0, 1]$

The following proposition compares equilibria with and without the friendship paradox in the simple case where there exist multiple (three) equilibria; one stable equilibrium at zero, and a strictly positive unstable and stable equilibria – as shown in Figure 4. This follows from standard assumptions about the shape of the CDF H of the benefit function that we assume for now, but later estimate empirically.

Proposition 1. *Let $E[a_{j:j \in N}]$ be the society-wide expected non-engagement and a_h and a_l be the non-zero symmetric Bayes Nash equilibrium non-engagement that solves Equation 2 such that $a_h > a_l$. Let $E[a_{j:j \in N(i)}]$ be the expected non-engagement among the agent's friends and let $\tilde{a}_l$ and $\tilde{a}_h$ be the symmetric Bayes Nash equilibria that solve Equation 3 such that $\tilde{a}_l < \tilde{a}_h$. Then, $\tilde{a}_l \leq a_l$ and $\tilde{a}_h \geq a_h$.*

³⁴The proof for Lemma 1 is provided in the appendix in Section G.2.

The proof is provided in Appendix Section G.3. Figure 4 illustrates this result. The proposition implies that in a society where there are three possible equilibrium values of non-engagement out of which two are non-zero, one stable, and one unstable, the highest stable equilibrium non-engagement is even higher when individuals observe the actions of their peers and not the entire society. The lower unstable equilibrium is even lower, implying that a larger shift in expected engagement is required to cross the tipping point and reach the zero equilibrium non-engagement.

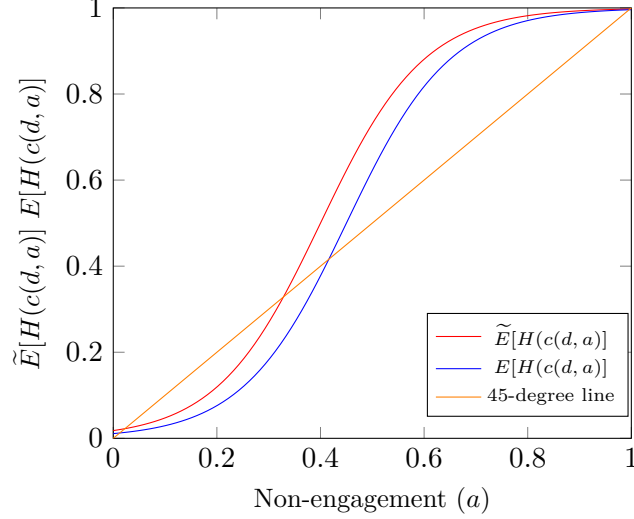


FIGURE 4. Illustration for Proposition 1 under standard assumptions about H .

7.3. Dynamics and the Effect of Belief Correction. Consider the dynamic form of the best response map of Equation 3:³⁵

$$\tilde{a}_t = \sum_d \tilde{P}(d)(H(c(d, \tilde{a}_{t-1})))$$

Information about the true share of those willing to engage shifts beliefs about $\tilde{a}_t$, which then affects engagement at $t+1$. This is useful in the case where individuals overestimate non-engagement because they naively proxy for the norm using the actions taken by their peers. Because individuals continue to infer norms from peers, the updated beliefs propagate recursively.

Importantly, we assume the intervention shifts the belief $\tilde{a}_t$ that individuals hold in a given period but does not change the *process* by which they form beliefs: in subsequent periods they continue to infer the norm naively from their peers, as in Equation 3. The information thus corrects the level of the belief without teaching individuals that proxying the norm from peers is itself biased, which is why effects can fade as the system reverts toward the peer-based equilibrium. The long-run effect depends on whether the information shock pushes the system past the unstable equilibrium. If not, the system gradually reverts to the original low-engagement equilibrium; if so, it converges to a higher-engagement one.

³⁵Previous period's non-engagement a_{t-1} is the relevant 'belief' that individuals have about others' willingness to not engage while deciding whether or not to engage in period t .

8. Structural Estimation and Longer-Term Effects

We structurally estimate the model using the RCT to (i) recover the equilibrium level of engagement in these communities and assess the scope for belief correction, and (ii) compare belief-shifting to alternative policy levers that do not target beliefs. We then assess whether outcomes measured two years later are consistent with the model’s predictions.

We make a few simplifying assumptions. First, the cost of engagement is $c(d, 1 - E[e_{j:j \in N(i)}]) = \theta * d * E[a_{j:j \in N(i)}]$ where $a_i = 1 - e_i$ is the choice to disengage. The benefit of engagement b_i is drawn from a log-logistic distribution H , implying $H(c(d, a)) = (1 + (\theta da/\alpha)^{-\kappa})^{-1}$ where we set $\alpha = 1$. This functional form allows for complete flexibility in the number and type of possible equilibria.³⁶ Figure H.1 plots the log-logistic function for different values of θ and κ to illustrate that we do not impose a specific shape.

Based on the evidence of a positive relationship between optimistic beliefs about peers’ willingness to engage and own engagement detected from the experiment results, we restrict the strategic complementarity parameter θ to be strictly positive. We restrict κ to be non-negative as the log-logistic distribution only allows a non-negative domain. The estimation recovers estimates of the two parameters that govern equilibrium selection: θ (complementarities in disengagement) and κ (shape of the benefit distribution).

We use simulated method of moments and proceed in the following steps.

1. Computation of Degree Distribution: The degree distribution $P(d)$ is computed using the overall network degree.³⁷ The peer degree distribution is: $\tilde{P}(d) = P(d) * (d/E(d))$, where the sample average degree is an estimate of the expected degree $E(d)$.³⁸

2. Equilibrium selection for the control group: For randomly picked initial θ and κ , we solve for the Bayes-Nash equilibria (Equation 3): $a^*(\theta, \kappa) = \sum \tilde{P}(d)H(c(d, a^*(\theta, \kappa)))$. Several equilibria may exist. Following Bisin et al. (2011) and De Paula (2013), we select the equilibrium that maximizes the likelihood of observed disengagement in the control group. We use a_c^* and d_i to compute the expected disengagement for each agent i in the control group. This is used to construct the first moment of interest: expected disengagement in the control group.

3. Treatment group prediction: We then compute the probability that each individual in the treatment group disengages. This probability depends on the exogenously delivered belief that

³⁶We use this distribution since its cumulative distribution function can be written in closed form, and this helps with accuracy and speed in our simulations. This is also a more suitable choice than the logistic distribution since it has a positive domain and satisfies $H(0) = 0$. The parameters of the logistic distribution only allow $H(0) = 0$ in the limiting case, and we show robustness to this in the Appendix.

³⁷Recall that the overall network contains a link between two agents if they interact in any capacity.

³⁸We do not compute degree distributions for each NGO center due to small samples in each center. There is no reason to believe that the networks should be systematically different across centers.

$a_t^* = 0.4$, the individual’s degree d_i , and the values of θ and κ .³⁹ This is used to construct the second moment of interest: expected disengagement in the treatment group.

4. Objective function: We minimize the squared deviation between model-predicted and empirical means of disengagement in the treatment and control groups, using pattern search with many starting points to avoid local minima.⁴⁰

We use willingness to participate in the listening service as the primary engagement measure, since this most closely corresponds to the information delivered. Robustness to alternative measures is discussed below.

8.1. Estimation Results. Table 1 reports the minimized objective function and shows that the estimated parameters fit the data closely. The model also fits the untargeted moments well—namely, the standard deviation and skewness of engagement—indicating good overall fit. Figure 5 plots the equilibrium equation $a = \sum_d \tilde{P}(d)(H(c(d, a); \theta, \kappa))$ evaluated at the estimated θ^* and κ^* .

The estimated best response curve implies three equilibria: two stable equilibria at $a^* = 0$ and $a^* = 0.89$, and an unstable equilibrium at $a^* = 0.23$. We find that the high-disengagement equilibrium at $a^* = 0.89$ maximizes the likelihood, consistent with the low baseline levels of engagement around such issues.⁴¹ Together, these results indicate that these communities are located near a high-disengagement steady state and that beliefs about disengagement must shift below the threshold $a^* = 0.23$ to generate persistent change.

TABLE 1. Structural Estimation Model Fit

Objective Function	Targeted Moments				Untargeted Moments	
	Predicted Mean (Treated)	Actual Mean (Treated)	Predicted Mean (Control)	Actual Mean (Control)	Standard Deviation (Squared Gap)	Skewness (Squared Gap)
0.0005	0.309	0.299	0.440	0.460	0.003	2.6e-05

Notes: This table shows the model fit for the estimated values of θ^* and κ^* for the main measure of engagement. The objective function is computed at the estimated parameters and is equal to the sum of the squared gap between the mean of the treatment and control groups predicted by the model and in the data respectively. For the untargeted moments, we report the squared gap between the model-predicted and actual values. As a result, all reported fit measures represent the model’s error using the same metric.

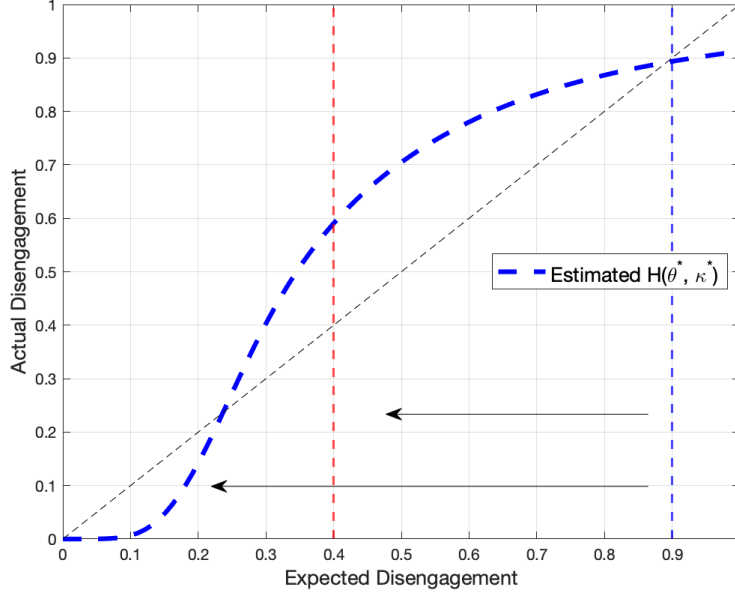
³⁹We do not have to solve for the equilibrium and pick the one that maximizes the likelihood in this case, since treated individuals were provided a value for the norm. Specifically, we assume that treated individuals interpret the delivered information as the proportion who would participate in the listening service. The same assumption is applied to the alternative engagement measures used for model estimation in Section 8.4.

⁴⁰We use the *pattern search* solver with multiple starting points to run these iterations and find the optimal θ and κ . This allows us to find the global optimum (Audet and Dennis Jr, 2002).

⁴¹This value is not directly comparable to the beliefs elicited in Section 3, which concern a general willingness to discuss mental-health concerns rather than engagement with specific activities.

Figure H.3 plots the estimated curve together with a confidence set. Section H.1 of the appendix describes the construction of the confidence set. The confidence set is tight, and the shape of the best response curve is similar for all the θ and κ values that lie in this set.

FIGURE 5. Structural Estimation: Actual and Expected Disengagement



Notes: The figure plots actual disengagement a and expected disengagement $\sum_d \tilde{P}(d)(H(c(d, a)))$ at the estimated θ^* and κ^* . The blue dotted line shows the equilibrium that maximizes the likelihood while the red dotted line shows the predicted short-run effect of the intervention.

The structural estimates allow us to assess whether the belief shift induced by the experiment is large enough to generate persistent change. Shifting perceived disengagement to 0.4—as in the RCT—does not cross the unstable equilibrium at $a^* = 0.23$. Because the community sits near the high-disengagement steady state, long-run change would require pushing perceived disengagement to lower than 20% (i.e., perceived engagement to be greater than 80%). Such a shift is substantial and may be difficult to deliver credibly in the status quo, where these topics are not openly discussed.

8.2. Effects after two years. The estimated model predicts that the intervention is too small to shift the community to a new equilibrium. We assess whether descriptive evidence from surveys conducted two years later is consistent with this prediction. We conducted a survey in the same setting in 2023 and classified individuals into four categories: (1) *Previous Participant* — reported participating in the 2020–21 surveys/experiment; (2) *Heard Community Beliefs* — reported hearing information about their community’s willingness to engage on mental-health and financial issues; (3) *Heard Community Beliefs + Previous Participant* — both participated earlier and reported hearing the information; and (4) *None of the above*.⁴² Although we cannot directly match individuals across waves, these self-reports enable us to identify likely exposure. Of 469 respondents, 60 fall into

⁴²This survey was conducted as part of the additional surveys completed in 2023 where we had an overlap of 1 center from the previous survey which we use to measure long-term effects.

Category 1, 37 into Category 2, and 24 into Category 3. We acknowledge that exposure is a proxy and may capture differences in individual characteristics apart from exposure to treatment. We therefore use post-double-selection Lasso (Belloni et al., 2014) to adjust for observable differences such as gender, income, age, and overall network degree. Unlike the experimental estimates from the RCT, the estimates in this subsection should be interpreted as descriptive long-run associations rather than causal treatment effects.

Tables 6–7 report the associations between these exposure categories and outcomes in 2023. Individuals in Categories 1–3 consistently hold more optimistic beliefs about others’ willingness to engage on mental-health and financial concerns, with effects strongest among those reporting that they heard the information.⁴³ They are also more willing to engage with peers and report significantly more interactions with peers around mental and financial well-being. Beyond network interactions, they report significantly higher happiness, lower consumption volatility, and fewer consumption crises. These patterns suggest that optimistic beliefs about peers increase network interactions and, in turn, improve psychological and economic outcomes.

While the evidence is only suggestive, it is notable that many relationships remain significant even under the most conservative measure of exposure (being a previous participant). However, it appears unlikely that beliefs and engagement have shifted permanently. For instance, among individuals reporting that they heard the information, average beliefs remain below the true value conveyed during the intervention. This is consistent with what the estimated model predicts: the belief shift was not sufficiently large to cross the tipping point.

8.3. Counterfactual Analysis. We now proceed to the counterfactual analysis to understand what types of policies can lead to a permanent shift in beliefs.

8.3.1. Counterfactual 1: Increasing the perceived benefits of interactions. We first consider policies that increase the perceived benefits of engagement—such as information or awareness sessions highlighting the value of network interactions, or financial incentives facilitating interactions. The top panel of Figure 7 plots the estimated best response curve evaluated at the estimated θ^* and κ^* and the dotted line in orange shows how much the curve must shift downward for the long-run effect to be at least as large as the short-run effect of the belief correction intervention.

Formally, shifting the curve is equivalent to increasing the benefits of engagement by a constant $b > 0$. As b rises, expected disengagement falls. We increase b until the resulting long-run equilibrium disengagement is no higher than 0.4, matching the short-run effect of the belief intervention.

We find that the mean benefit must increase by roughly 48% to generate a persistent long-run impact of a comparable magnitude, which is a substantial increase. Pushing benefits even higher would collapse disengagement to zero.

8.3.2. Counterfactual 2: Reducing how much agents care about the social norm. A second class of policies could lower θ , the extent to which violating social norms raises the costs

⁴³We drop respondents who reported hearing community views as part of the current 2023 surveys.

of engagement. Such policies could include psychological interventions that reduce concerns about gossip or judgment, or institutional changes—such as a formal job referral service—that lessen the signaling consequences of seeking help.

We therefore ask: by how much must θ fall to generate a long-run reduction in disengagement at least as large as the RCT’s short-run effect? The bottom panel of Figure 7 shows the resulting counterfactual. We gradually decrease θ and compute the implied equilibrium until disengagement falls to 0.4 or lower. We find that θ must fall by roughly 33% relative to the estimated θ^* . This is a sizable reduction, implying that policies aimed at lowering norm-related costs would need to be substantial—and likely costly—to match the short-run impact of belief correction.

Overall, our counterfactual analysis shows that generating long-run effects through policies that shift the benefits or costs of engagement can be very demanding. By comparison, belief correction produces sizable short-run effects at low cost and can help increase support for more resource-intensive policies.

8.4. Robustness to other measures of engagement and distributional assumptions.

Finally, we assess the robustness of our estimation strategy using two alternative measures of engagement: (1) willingness to join a savings group and (2) a composite ‘community engagement’ index (described earlier). Although the treatment information concerned willingness to engage with mental health topics, these outcomes are still plausible proxies for engagement; nevertheless, participation in the listening service remains the closest measure. For both alternative outcomes, we obtain a similar S-shaped best-response curve with three equilibria and a comparable belief threshold (Appendix Table H.1). This implies that sufficiently strong belief shifts would generate long-run change across all measures.

We also examine robustness to the assumed benefit distribution by replacing the log-logistic with a logistic distribution. Re-estimating the model now requires three parameters— θ , the mean κ , and the standard deviation σ —with σ identified using the empirical standard deviation of engagement. As shown in Figure H.4 in the appendix, the resulting best-response curve and implied equilibria are nearly identical to those under the log-logistic specification.

9. Conclusion

Informal safety nets cannot function effectively if individuals do not demand support. While this concern is relevant across a variety of settings ranging from risk-sharing in communities to advice-taking in organizations, it is especially consequential in environments where individuals lack formal assistance. In such an environment, we show that inaccurate beliefs about others’ willingness to engage around financial and mental-health concerns can trap communities in a low-engagement equilibrium even when peers are willing to engage.

Using a field experiment in urban informal communities in India, we demonstrate that correcting these misperceptions increases the demand for network-based support, consistent with lowering perceived reputational and interaction costs. Individuals with pessimistic beliefs place the highest weight on these social costs while deciding whether an interaction would take place. Additional

experiments confirm these mechanisms and help rule out alternative explanations, including social desirability concerns.

To understand why misperceptions arise and whether belief correction can shift communities to a new equilibrium, we adapt and structurally estimate a network model in which individuals infer norms from their peers' behavior. Structural estimates suggest that belief correction alone is unlikely to durably shift communities out of a low-engagement equilibrium. Consistent with this, suggestive evidence after two years shows that respondents reporting prior exposure have more optimistic beliefs and better outcomes, although their beliefs remain below the levels conveyed. Counterfactuals suggest that policies capable of generating persistent change are considerably more demanding. Nonetheless, belief correction can increase short-run demand for engagement within networks and help generate resources to finance more intensive interventions, a useful feature in underfunded settings.

Future work could investigate whether repeated belief correction can generate the sustained shifts needed for equilibrium change, and whether combining belief interventions with platforms that facilitate interaction, such as savings groups or community listening services, can reinforce short-run gains into durable improvements in informal safety nets.

Tables and Figures

A. Summary Statistics.

TABLE 2. Summary Statistics

	Mean	SD
<i>A. Demographics</i>		
Age	33.75	(9.371)
Female	0.352	(0.478)
Monthly HH Income (< Rs 2,500)	0.157	(0.364)
Monthly HH Income (Rs 2,500-5,000)	0.189	(0.392)
Monthly HH Income (Rs 5,000-10,000)	0.444	(0.498)
Monthly HH Income (> Rs 10,000)	0.210	(0.408)
Employed (Yes/No)	0.668	(0.472)
<i>B. Well-Being</i>		
Stress (Index; Scale 1-5)	3.076	(0.864)
Unable to manage difficulties (Often/Very Often)	0.503	(0.501)
Life Satisfaction (Scale 1-4)	2.851	(0.951)
Happiness (Scale 1-4)	2.479	(0.981)
<i>C. Networks</i>		
Number of network connections (Advice)	2.787	(3.378)
Number of network connections (Overall)	3.954	(3.945)
Observations	352	

Notes: The above table shows the summary statistics (mean and standard deviation) for various demographic characteristics of interest from the baseline sample.

TABLE 3. Summary Statistics

	Mean	SD
<i>A. Willingness to Engage</i>		
Willingness to Talk (Financial Concerns)	0.707	(0.456)
Willingness to Talk (Mental Health)	0.631	(0.483)
Willingness to Talk (Physical Health)	0.618	(0.487)
Stigma (Physical Health)	0.555	(0.498)
Stigma (Mental Health)	0.503	(0.501)
<i>B. Beliefs</i>		
Beliefs (Mental Health)	4.985	(3.012)
Beliefs (Physical Health)	5.126	(3.029)
Beliefs (Financial Concerns)	5.960	(3.196)
Beliefs -Stigma (Mental Health)	4.802	(3.312)
Beliefs -Stigma (Physical Health)	4.812	(3.253)
Beliefs -Stress (Mental Health)	5.641	(3.313)
<i>C. Frequency of Conversations</i>		
Dialogue (Physical Health; 1-5)	2.180	(1.300)
Physical Health talk (Never)	0.430	(0.496)
Physical Health talk (Rarely)	0.223	(0.417)
Physical Health talk (Sometimes)	0.162	(0.369)
Physical Health talk (Often)	0.110	(0.313)
Physical Health talk (Very Often)	0.0762	(0.266)
Dialogue (Mental Health; 1-5)	2.511	(1.304)
Mental Health talk (Never)	0.292	(0.456)
Mental Health talk (Rarely)	0.252	(0.435)
Mental Health talk (Sometimes)	0.191	(0.394)
Mental Health talk (Often)	0.182	(0.386)
Mental Health talk (Very Often)	0.0831	(0.276)
Dialogue (Financial Concerns; 1-5)	2.847	(1.470)
Financial Concerns talk (Never)	0.256	(0.437)
Financial Concerns talk (Rarely)	0.200	(0.401)
Financial Concerns talk (Sometimes)	0.181	(0.386)
Financial Concerns talk (Often)	0.166	(0.372)
Financial Concerns talk (Very Often)	0.197	(0.398)
Observations	334	

Notes: The above table shows summary statistics (mean and standard deviation) for additional baseline variables such as willingness to engage, dialogue intensity, stigma, and beliefs about peers. All variables in Panel A are binary. We measure stigma by asking if individuals think people should “stay away” from those with specific health concerns. Beliefs about peers are measured in terms of 0-10 individuals in the community. Dialogue intensity is measured on a scale from 1-5 where 1 is ‘Never’ and 5 is ‘Very Often’.

B. Endline Results.

TABLE 4. Treatment Effect on Endline Outcomes

Panel A: Treatment Effects on Community Engagement

Variables	Savings Group	Listening Volunteer	Listening Contribution	Contribution (in Rupees)	Community Engagement
Treatment	0.147** (0.0711)	0.161** (0.0730)	0.122** (0.0612)	6.577** (2.959)	0.138*** (0.0516)
<i>Bootstrap p-value</i>	0.0831	0.0511	0.0561	0.0931	0.0340
<i>q-values</i>	0.046	0.046	0.046	0.046	0.046
Constant	0.667*** (0.0548)	0.540*** (0.0537)	0.738*** (0.0483)	22.72*** (2.090)	0.662*** (0.0393)
Observations	150	174	170	163	150
R-squared	0.028	0.027	0.023	0.030	0.046

Panel B: Treatment Effects on Own Health Outcomes

	Speaking to the Doctor (MH)	Depression Scoring (Immediate)	Depression Scoring	Listening to Helpline Numbers
Treatment	0.0995* (0.0600)	-0.272*** (0.0809)	-0.136* (0.0747)	-0.197** (0.0844)
<i>Bootstrap p-value</i>	0.232	0.0300	0.469	0.0511
<i>q-values</i>	0.164	0.008	0.224	
Constant	0.118*** (0.0373)	0.606*** (0.0584)	0.789*** (0.0488)	0.591*** (0.0610)
Observations	154	143	143	137
R-squared	0.018	0.074	0.023	0.039

Panel C: Treatment Effects on Other Endline Outcomes

VARIABLES	Memory (Numbers Remembered)	Physical Health Dialogue (with Family)
Treatment	-0.271 (0.205)	0.208*** (0.0711)
<i>Bootstrap p-value</i>	0.387	0.0210
Constant	1.053*** (0.151)	0.184*** (0.0448)
Observations	153	155
R-squared	0.011	0.053

Notes: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$. Robust standard errors in parentheses. Wild bootstrap p -values reported (reps=999) using the method outlined in Cameron et al. (2008) where we treat the NGO center as the cluster unit. Definitions of all outcomes are provided in Appendix Section C.1.

C. Longer Term Effects.

TABLE 5. Effects on Network Interactions after 2 weeks

	Financial Concerns			Mental Health Concerns		
	Reached Out (Yes/No)	Number of Friends Reached Out to	Friend Reached Out (Yes/No)	Reached Out (Yes/No)	Number of Friends Reached Out to	Friend Reached Out (Yes/No)
Treatment	0.188** (0.0930)	1.212*** (0.427)	0.146 (0.0914)	0.113 (0.0863)	0.911** (0.386)	0.0414 (0.0833)
<i>q-values</i>	0.052	0.034	0.054	0.034	0.008	0.139
Constant	0.321*** (0.0647)	0.877*** (0.206)	0.283*** (0.0625)	0.226*** (0.0580)	0.500*** (0.158)	0.226*** (0.0580)
Observations	110	109	109	109	109	109
R-squared	0.036	0.068	0.023	0.016	0.048	0.002

Notes: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$. Robust standard errors in parentheses. The *q-values* (Benjamini et al., 2006) reported in each table treat all the outcomes in this table as multiple hypotheses being tested together. Standard errors are robust.

TABLE 6. Effects on Beliefs and Willingness to Engage after 2 years

<i>Panel A: Effects on Beliefs about Peers after 2 years</i>						
VARIABLES	Beliefs (MH)	Beliefs (MH)	Beliefs (MH)	Beliefs (FC)	Beliefs (FC)	Beliefs (FC)
Previous Participant	0.193 (0.299)			0.00819 (0.261)		
Heard Information		0.899*** (0.324)			0.751*** (0.259)	
Heard Information and was previous participant			1.442*** (0.387)			0.754** (0.307)
Constant	3.607*** (0.0869)	3.560*** (0.0866)	3.558*** (0.0850)	3.658*** (0.0930)	3.600*** (0.0915)	3.621*** (0.0898)
Observations	467	467	467	467	467	467
<i>Panel B: Effects on Willingness to Engage after 2 years</i>						
VARIABLES	Willingness to Engage (MH)	Willingness to Engage (MH)	Willingness to Engage (MH)	Willingness to Engage (FC)	Willingness to Engage (FC)	Willingness to Engage (FC)
Previous Participant	0.0185 (0.0448)			0.0611* (0.0362)		
Heard Information		0.0561 (0.0478)			0.0715* (0.0405)	
Heard Information and was previous participant			0.140*** (0.0165)			0.126*** (0.0158)
Constant	0.865*** (0.0169)	0.863*** (0.0166)	0.860*** (0.0165)	0.872*** (0.0165)	0.874*** (0.0160)	0.874*** (0.0158)
Observations	467	467	467	467	467	467

Note: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$. Robust standard errors in parentheses. We implement post double selection lasso (Belloni et al., 2014) with robust standard errors accounting for income, age, gender, and number of network connections. Previous participant is a binary variable equal to 1 if the respondent in 2023 reports being contacted for the previous study in 2020-21. Heard information is equal to 1 if they report having heard anything about their community's willingness to engage from their peers and 0 otherwise. "MH" refers to mental health concerns and "FC" refers to financial concerns.

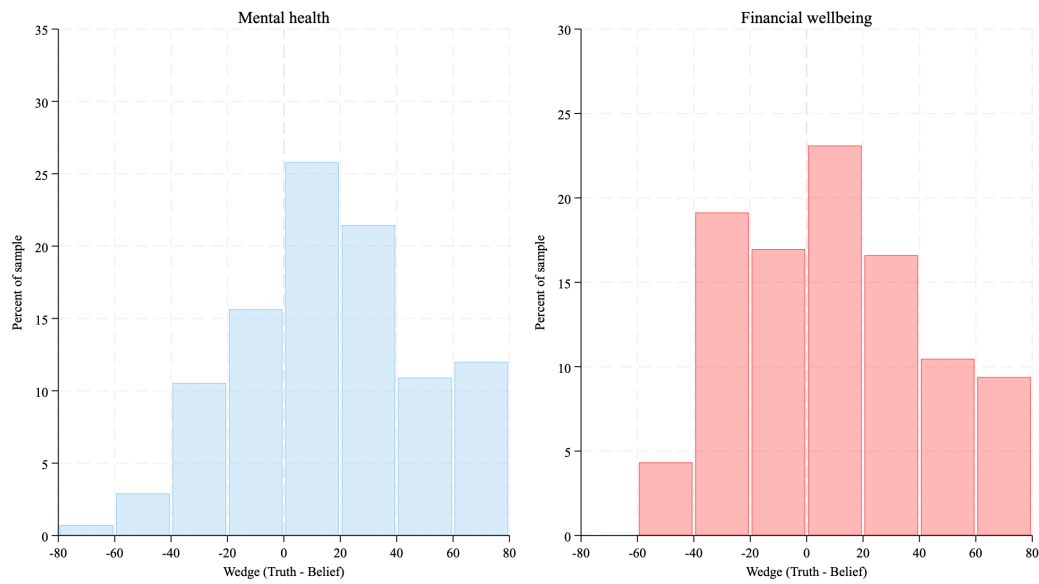
TABLE 7. Effects on Network Interactions and Consumption Outcomes after 2 years

<i>Panel A: Effects on Network Interactions after 2 years</i>						
VARIABLES	(1) MH Talk (Above Median)	(2) MH Talk (Above Median)	(3) MH Talk (Above Median)	(4) FC Talk (Above Median)	(5) FC Talk (Above Median)	(6) FC Talk (Above Median)
Previous Participant	0.123* (0.0689)			0.0814 (0.0685)		
Heard Information		0.183** (0.0841)			0.263*** (0.0830)	
Heard Information and was previous participant			0.210** (0.102)			0.171 (0.104)
Constant	0.410*** (0.0244)	0.412*** (0.0237)	0.415*** (0.0234)	0.369*** (0.0239)	0.358*** (0.0231)	0.370*** (0.0229)
Observations	467	467	467	467	467	467
<i>Panel B: Effects on Consumption Outcomes after 2 years</i>						
VARIABLES	Consumption Crisis (Yes/No)	Consumption Crisis (Yes/No)	Consumption Crisis (Yes/No)	Consumption varies a lot	Consumption varies a lot	
Previous Participant	0.0124 (0.0690)			-0.0649** (0.0275)		
Heard Information		-0.157* (0.0833)			-0.0683** (0.0302)	
Heard Information and was previous participant			-0.155 (0.102)			
Constant	0.521*** (0.0248)	0.535*** (0.0241)	0.530*** (0.0237)	0.0983*** (0.0148)	0.0953*** (0.0142)	
Observations	467	467	467	467	467	
<i>Panel C: Effects on Self-Reported Happiness after 2 years</i>						
VARIABLES	Happiness	Happiness	Happiness			
Previous Participant	0.601*** (0.169)					
Heard Information		0.818*** (0.180)				
Heard Information and was previous participant			0.675*** (0.240)			
Constant	3.115*** (0.0582)	3.128*** (0.0573)	3.158*** (0.0566)			
Observations	467	467	467			

Note: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$. Robust standard errors in parentheses. We implement post double selection lasso (Belloni et al., 2014) with robust standard errors accounting for income, age, gender, and number of network connections. *Previous participant* is a binary variable equal to 1 if the respondent in 2023 reports being contacted for the previous study in 2020-21. *Heard information* is equal to 1 if they report having heard anything about their community's willingness to engage from their peers and 0 otherwise. "MH" refers to mental health, and "FC" refers to financial concerns.

D. Misperception Distribution.

FIGURE 6. Distribution of the Belief Wedge



Distribution of the belief wedge (percentage points) in the sample.

Notes: Each panel plots the distribution of the belief wedge (truth minus own belief, in percentage points) for the full sample. Negative values indicate overestimation; positive values indicate underestimation of peers' willingness to engage.

E. Mechanisms.

TABLE 8. Hypothetical Network Prediction Experiment (OLS)

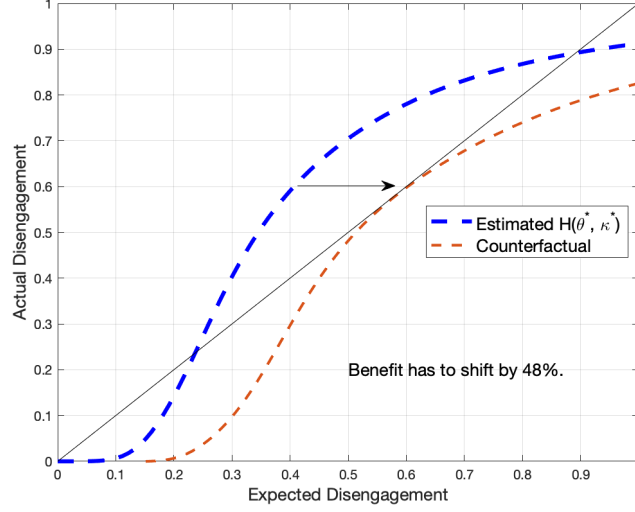
VARIABLES	(1) B takes advice (MH)	(2) B takes advice (MH)	(3) B takes advice (FC)	(4) B takes advice (FC)
Advisor has job contacts	0.00851 (0.0174)	0.00687 (0.0173)	0.0249 (0.0177)	0.0240 (0.0176)
Advisor is network central	0.0100 (0.0174)	0.00740 (0.0173)	0.0222 (0.0177)	0.0233 (0.0176)
Advisor has attended training	-0.0181 (0.0174)	-0.0182 (0.0173)	-0.00929 (0.0177)	-0.00972 (0.0176)
Advisor has job contacts X Beliefs about Community		0.0226 (0.0183)		0.0158 (0.0173)
Advisor is network central X Beliefs about Community		0.0347* (0.0184)		0.0354** (0.0173)
Advisor has attended training X Beliefs about Community		0.0312* (0.0183)		0.0482*** (0.0173)
Beliefs about Community (MH)		-0.00548 (0.0184)		
Beliefs about Community (FC)				-0.0635*** (0.0176)
Constant	0.766*** (0.0173)	0.767*** (0.0172)	0.737*** (0.0178)	0.737*** (0.0176)
Observations	2,372	2,372	2,370	2,370
R-squared	0.001	0.013	0.002	0.008

Notes: *** $p < 0.01$, ** $p < 0.05$, * $p < 0.1$. Robust standard errors in parentheses. This table presents the results of the link prediction experiment where individuals are asked to predict if a link exists between a hypothetical advisor (A) and a randomly chosen person (B). "MH" refers to mental health and "FC" refers to financial concerns. "Beliefs" refers to the respondent's prediction (standardized) about how many community members would be willing to engage around mental or financial concerns. The interaction terms interact the advisor characteristics with Beliefs (MH) in Column 2 and Beliefs (FC) in Column 4. We report robust standard errors.

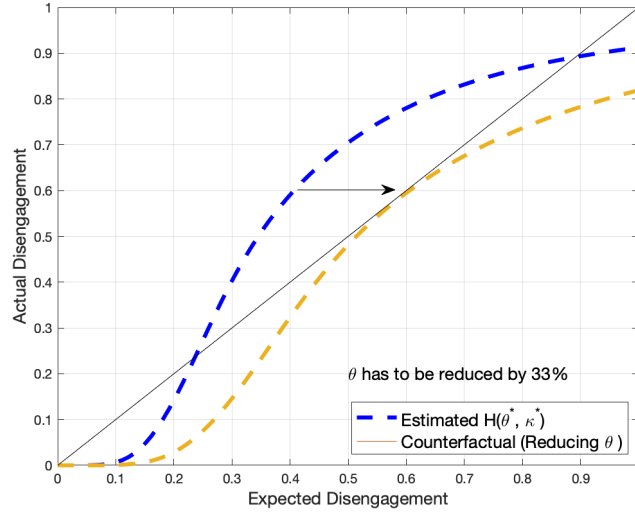
F. Structural Estimation: Counterfactual Results.

FIGURE 7. Counterfactual Interventions

(A) Increasing the Benefit of Engagement.



(B) Reducing how much agents care about the Social Norm.



Notes: The figures plot actual disengagement a and expected disengagement $\sum_d \tilde{P}(d)(H(c(d, a)))$ at the estimated θ^* and κ^* in blue. In addition to this, the figures also plot the proposed counterfactuals that increase benefits (top) and reduce θ (bottom) in the dotted line in orange/yellow.

Pre-registration and pre-analysis plan

The main experiment conducted in 2021 was pre-registered at the AEA RCT registry: <https://doi.org/10.1257/rct.6980-2.1> before the analysis began. The main outcome families and multiple-testing corrections reported in the paper follow the PAP. We note the following differences between the PAP and the presentation in the paper:

- (1) The PAP included measures of respondents' preferences over whom they would choose to talk to about mental-health concerns. We conducted these analyses and find no statistically significant treatment effects. We do not report them in the paper due to space constraints and would be happy to provide them if needed.
- (2) The PAP specified a broader set of heterogeneity analyses than those reported in the paper. We conducted these analyses, but present only the dimensions most closely related to the mechanisms discussed in the paper. The omitted heterogeneity results do not alter the conclusions and we would be happy to provide them if needed. Conversely, Tables C.5 and C.7 examine heterogeneity by baseline mental-health stigma and beliefs about community stress, respectively; these heterogeneity dimensions were not separately listed in the PAP, but help us understand the mechanisms.
- (3) The PAP specified willingness to undertake depression scoring and willingness to listen to good practices as outcomes. In the paper, we distinguish between immediate and overall take-up for both measures. Immediate take-up measures whether respondents agreed to complete the activity straight away, while overall take-up also includes respondents who agreed to complete it during a later call. We additionally report willingness to listen to information about government mental-health helplines; this outcome was not separately listed in the PAP but helps us measure how the demand for formal support changes.
- (4) The PAP specified standard errors clustered at the community level. Given the small number of clusters in our sample, we instead report robust standard errors together with wild-cluster bootstrap p-values (Cameron et al., 2008), clustering at the community level. Wild-cluster bootstrap inference is better suited to settings with few clusters and is our preferred basis for inference allowing for within-community correlation.
- (5) The supplementary 2023 information experiment was conducted subsequently as a supplementary exercise to explore mechanisms and validate the patterns of the original experiment, and was not part of the original 2021 PAP. It measured the same main engagement outcomes as the 2021 experiment: willingness to join a savings group, willingness to participate in the listening service, and willingness to make financial contributions toward these activities, which are discussed in the appendix.

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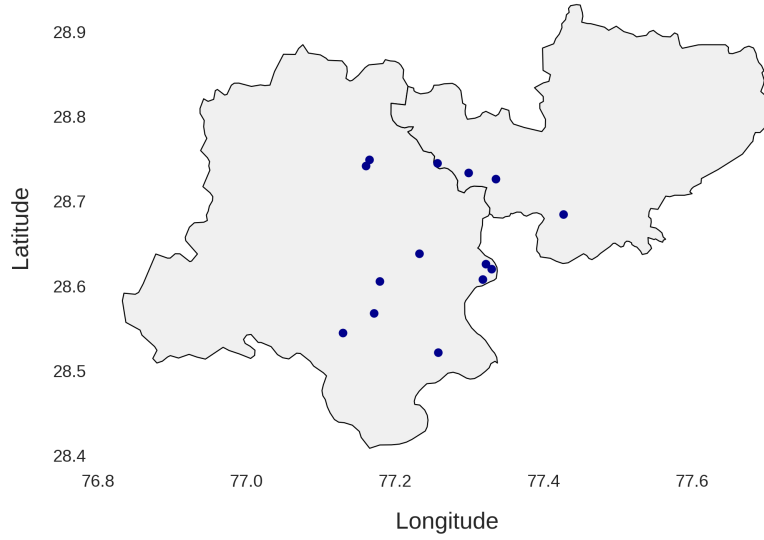
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Supplementary Appendix

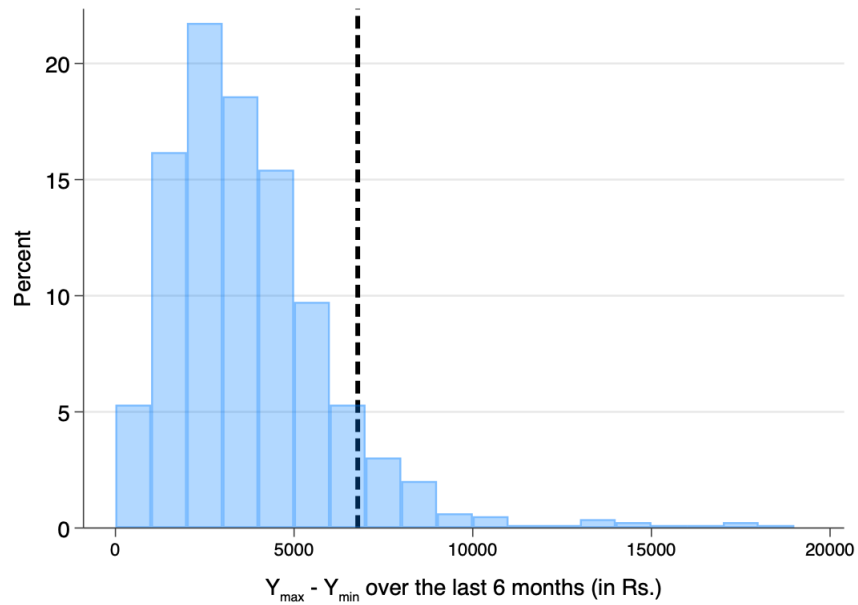
A. Baseline.

FIGURE A.1. Survey Locations in the National Capital Region



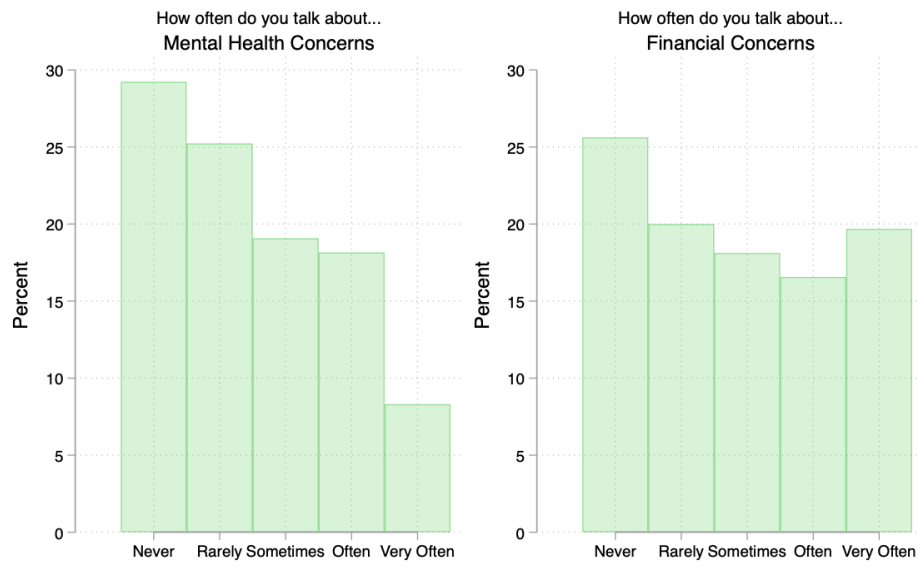
Notes: The figure plots the locations of various centers where we conducted the surveys (in blue) on the map of the national capital region.

FIGURE A.2. Volatility of Incomes across 6 months



Notes: The figure plots the difference between the maximum and minimum income an individual has earned in the last six months (in Rs.). The dashed line is the average current income in the sample. This data was collected for the additional sample in 2023.

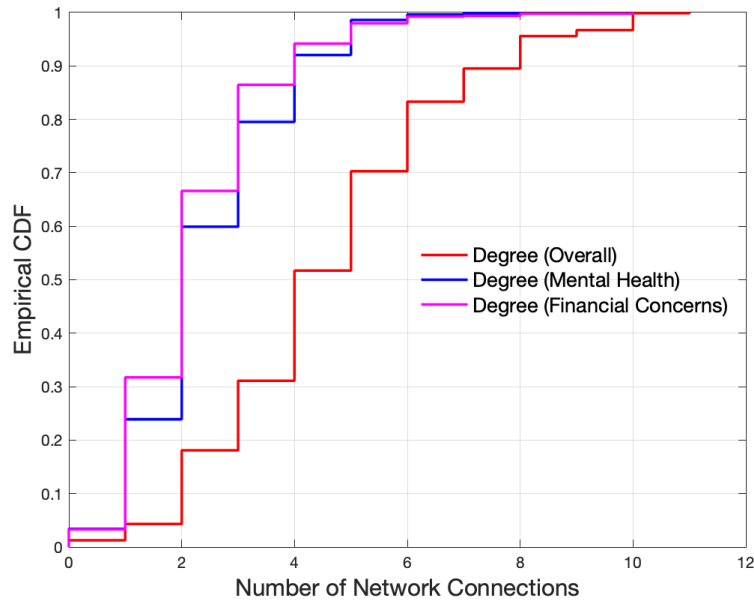
FIGURE A.3. Intensity of Dialogue in the last two weeks



Graphs by Topic

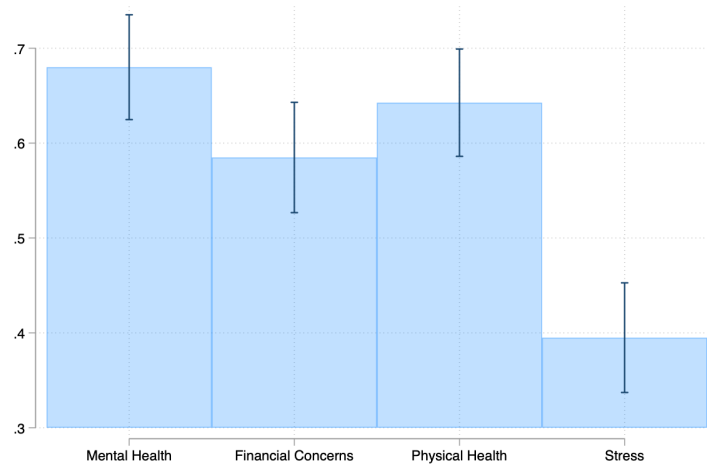
Notes: The figure plots the baseline intensity of dialogue around mental health and financial concerns with peers in the last two weeks.

FIGURE A.4. Degree Distributions for Overall Networks, Mental Health Advice Networks, and Financial Networks in 2023



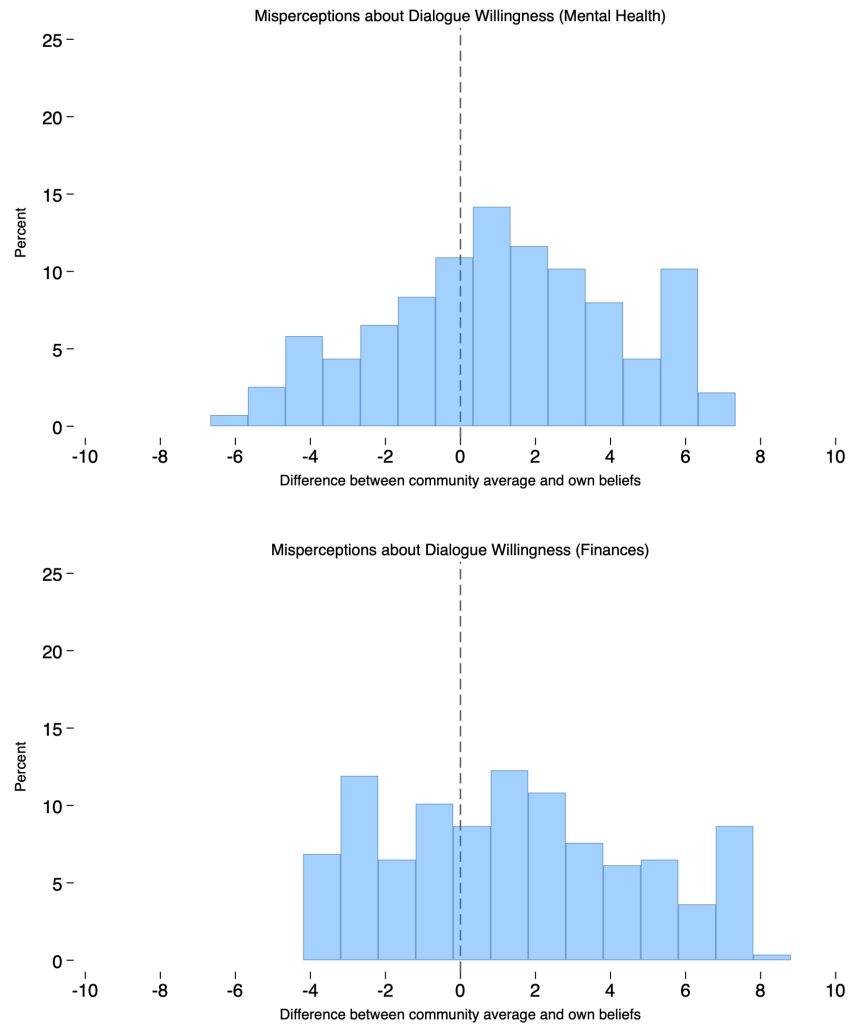
Notes: This figure plots the CDF of the number of connections in the overall networks, advice networks, and financial networks for borrowing and lending. Individuals were asked to list up to ten other individuals in their community whom they interact with to borrow/lend, take/give advice, work with etc. (“Overall”), whom they only take advice from regarding mental health issues (“Mental Health”), and whom they contact for financial support (“Financial Concerns”).

FIGURE A.5. Percentage of Underestimators



Notes: This figure plots the proportion of underestimators by type of dialogue and additionally by “Stress” where individuals are asked to predict how many others they think would say that their “difficulties were piling up so high that they could not overcome them” often or very often. Underestimators are identified by comparing their beliefs about community’s willingness to engage with actual average in the community (i.e., NGO center).

FIGURE A.6. Misperceptions about Community Willingness to Engage



Notes. Each panel plots the difference between the actual willingness to engage in an individual's own community and that individual's own belief, on a 0 to 10 scale. Positive values mean the individual underestimates how willing their community is to engage. The top panel is for mental health and the bottom panel is for financial concerns.

TABLE A.1. Correlations among Beliefs and Engagement along various dimensions

	Beliefs (FC)	Beliefs (PH)	Beliefs (MH)	Willing to engage (FC)	Willing to engage (PH)	Willing to engage (MH)
Beliefs (FC)	1					
Beliefs (PH)	0.591***	1				
Beliefs (MH)	0.588***	0.496***	1			
Willing to engage (FC)	0.151*	0.172**	0.206***	1		
Willing to engage (PH)	0.0586	0.0860	0.181**	0.446***	1	
Willing to engage (MH)	0.0516	0.137*	0.106	0.440***	0.550***	1

* $p < 0.05$, ** $p < 0.01$, *** $p < 0.001$

Note: This table presents correlations between beliefs and willingness to engage along various topics. The entry in the i 'th row and j 'th column reports the correlation coefficient between the i 'th and j 'th variable. "MH" refers to mental health, "PH" refers to physical health, and "FC" refers to financial concerns. Beliefs refer to how many community members out of any 10 individuals believe would be willing to engage around a topic. Willing to engage is a binary variable equal to 1 if they themselves are willing to engage with others around a topic.

TABLE A.2. Correlations of Network Gaps and Dialogue Intensity

<i>Panel A: Correlations with Beliefs</i>				
VARIABLES	Degree Gap	Dialogue (MH)	Dialogue (PH)	Dialogue (FC)
Beliefs (MH)	-0.015 (0.011)	0.064* (0.033)		
Beliefs (PH)			0.021 (0.024)	
Beliefs (FC)				0.085*** (0.021)
Observations	210	271	275	263
R-squared	0.015	0.022	0.002	0.034
<i>Panel B: Correlations with Underestimation</i>				
VARIABLES	Degree Gap	Degree Gap	Dialogue (MH)	Dialogue (MH)
Underestimator	-0.015 (0.065)		-0.337* (0.169)	
Severe Underestimator		0.178** (0.070)		-0.373* (0.183)
Observations	210	210	271	271
R-squared	0.000	0.039	0.015	0.015

Note: This table reports regression results where we regress degree gaps and dialogue intensity on (a) various measures of beliefs and (b) whether an individual underestimates engagement with mental health. *Underestimator* is equal to 1 if an individual underestimates how many others in their community are willing to engage. *Severe Underestimator* is equal to 1 if the difference between actual willingness to engage and their belief is greater than the 75th percentile. We construct "Degree Gap" as a network-based measure of an individual's disengagement using differences in degrees in overall and advice networks. "MH" refers to mental health, "PH" refers to physical health, and "FC" refers to financial concerns. Standard errors are robust and clustered at the center level.

TABLE A.3. Correlations of Willingness to Engage with Beliefs about Community.

VARIABLES	(1) Willing to have MH Dialogue	(2) Willing to have MH Dialogue	(3) Willing to have PH Dialogue	(4) Willing to have PH Dialogue
Beliefs (MH)	0.016 (0.012)			
Beliefs -Stigma (MH)		-0.012* (0.006)		
Beliefs (PH)			0.014 (0.010)	
Beliefs -Stigma (PH)				-0.023** (0.008)
Observations	274	277	275	276
R-squared	0.010	0.007	0.007	0.023

Notes: This table reports results where we regress willingness to engage in dialogue on beliefs about peers' willingness to engage and beliefs about stigma among peers. 'MH' refers to mental health, 'PH' refers to physical health, and 'FC' refers to financial concerns. Standard errors are robust and clustered at the level of the center.

TABLE A.4. Correlation between Beliefs and Demographic/Network Characteristics
(in 2023)

VARIABLES	(1)	(2)
	Belief (MH)	Belief (FC)
Male	-0.109 (0.132)	0.0199 (0.139)
Age	-0.000661 (0.00599)	0.00437 (0.00607)
Income	4.22e-05* (2.32e-05)	1.15e-05 (2.33e-05)
Degree (Overall)	-0.150*** (0.0510)	-0.171*** (0.0510)
Degree (MH)	0.265*** (0.0664)	0.129* (0.0681)
Degree (FC)	0.351*** (0.0609)	0.312*** (0.0591)
Talks to peers (MH)	0.0708 (0.153)	0.208 (0.182)
Talks to peers (FC)	-0.177 (0.146)	-0.0771 (0.174)
Years in Location	-0.00832 (0.00532)	0.00474 (0.00583)
Consumption Crisis	0.277* (0.142)	0.167 (0.147)
Volatile Consumption	-0.405*** (0.134)	-0.113 (0.138)
Happiness	0.204*** (0.0608)	0.342*** (0.0632)
Constant	2.512*** (0.393)	1.798*** (0.393)
Observations	775	775
R-squared	0.109	0.085
Robust standard errors in parentheses		
*** p<0.01, ** p<0.05, * p<0.1		

Notes: This table presents the regression results where we regress beliefs about community members' willingness to engage around mental health concerns and financial concerns on demographic characteristics, network characteristics, dialogue intensity, and economic indicators. "MH" refers to mental health concerns and "FC" refers to financial concerns. Talks to peers (MH)/ Talks to peers (FC) is a dummy equal to 1 if the individual reports having spoken about mental health or financial concerns to their peers often/very often in the last 2 weeks. Degree refers to number of connections. Happiness is defined on a scale of 1-5. Volatile consumption is a dummy equal to 1 when the individual reports that their consumption has fluctuated a little/a lot over the last 6 months. Consumption crisis is a dummy equal to 1 if the individual reports having faced consumption crisis events very often (i.e., not having enough financial resources to meet basic dietary needs, education expenditures, or health expenditures).

B. Design.

B.1. Endline Balance.

TABLE B.1. Endline Balance: Demographics and Health

	Control	Treatment	p-value
Age	34.69	34.66	0.98
Female	0.30	0.34	0.60
Income (Category)	2.81	2.83	0.89
Monthly HH Income (< Rs 2,500)	0.14	0.11	0.48
Monthly HH Income (Rs 2,500-5,000)	0.16	0.22	0.30
Monthly HH Income (Rs 5,000-10,000)	0.44	0.41	0.65
Monthly HH Income (Rs 10,000+)	0.26	0.27	0.89
Employed	0.67	0.68	0.91
Stress (Index; Scale 1-5)	3.10	3.07	0.82
Satisfaction (Scale 1-4)	2.84	2.94	0.47
Happiness (Scale 1-4)	2.41	2.48	0.65
Degree (Advice)	2.59	3.14	0.28
Degree (Overall)	3.84	4.57	0.24
Degree (Advice Network) >0	4.67	4.76	0.88
Degree (Overall Network) >0	5.79	5.91	0.85
<i>N</i>	92	88	

Notes: This table presents the results of balance tests between the treatment and control group for the endline sample where the first two columns show the means of the variables for the Control and Treatment Group and the third column shows the p -values for the two-sided test that the two means are equal using robust standard errors. *Degree* refers to the number of connections and *Degree* (> 0) only includes strictly positive entries.

TABLE B.2. Endline Balance: Willingness to Talk and Beliefs

	Control	Treatment	p-value
Willingness to Talk (Financial)	0.70	0.74	0.59
Willingness to Talk (Mental Health)	0.67	0.56	0.12
Willingness to Talk (Physical Health)	0.60	0.63	0.68
Stigma (Physical Health)	0.63	0.53	0.21
Stigma (Mental Health)	0.49	0.50	0.94
Beliefs (Mental Health)	4.70	5.18	0.33
Beliefs (Physical Health)	5.08	5.26	0.73
Beliefs (Financial Concerns)	5.59	6.71	0.03
Beliefs -Stigma (Mental Health)	4.84	5.05	0.70
Beliefs -Stigma (Physical Health)	4.95	5.05	0.84
Beliefs -Stress (Mental Health)	5.67	6.36	0.20
Dialogue (Physical Health; 1-5)	2.22	2.08	0.51
Physical Health talk (Never)	0.45	0.47	0.78
Physical Health talk (Rarely)	0.18	0.19	0.88
Physical Health talk (Sometimes)	0.16	0.19	0.59
Physical Health talk (Often)	0.11	0.07	0.34
Physical Health talk (Very Often)	0.09	0.07	0.64
Dialogue (Mental Health; 1-5)	2.50	2.39	0.58
Mental Health talk (Never)	0.34	0.33	0.87
Mental Health talk (Rarely)	0.17	0.30	0.05
Mental Health talk (Sometimes)	0.21	0.13	0.19
Mental Health talk (Often)	0.21	0.14	0.27
Mental Health talk (Very Often)	0.07	0.10	0.53
Dialogue (Financial Concerns; 1-5)	2.81	3.01	0.37
Financial Concerns talk (Never)	0.27	0.20	0.28
Financial Concerns talk (Rarely)	0.18	0.21	0.56
Financial Concerns talk (Sometimes)	0.18	0.18	1.00
Financial Concerns talk (Often)	0.20	0.18	0.70
Financial Concerns talk (Very Often)	0.17	0.23	0.33
<i>N</i>	92	88	

Notes: This table presents additional results of balance tests between the treatment and control group for the endline sample where the first two columns show the means of the variables for the Control and Treatment Group and the third column shows the p -values for the two-sided test that the two means are equal using robust standard errors. All variables are binary except beliefs and dialogue intensity. Beliefs are measured in terms of 0-10 individuals in the community. Dialogue intensity is measured on a scale from 1-5 where 1 is ‘Never’ and 5 is ‘Very Often’.

C. Endline Results.

C.1. Outcome Definitions and Hypotheses.

Community Engagement

- **Savings Group.** Whether the individual is willing to join an NGO savings group where members pool money to help each other in hard times. The exact wording was “We are thinking of starting a savings group with the permission of the NGO in which you and your community members can discuss financial concerns openly and can assemble money to help each other in difficult times. Would you like to be a part of such a group?”
- **Listening Volunteer.** Whether the individual is willing to train as a volunteer who listens to the worries of other community members. The exact wording was “We want some members of the community to take out time to listen to the worries of other members and become mental health companions. They will be trained before this but will not receive any money. Do you wish to become a mental health companion?”
- **Listening Contribution.** Whether the individual is willing to donate part of a possible Rs 50 prize to help set up the listening service. The exact wording was “If you win this prize, then would you be willing to donate some of this to organize the above training in your community?”
- **Contribution (in Rupees).** How much of the Rs 50 prize the individual is willing to donate, and Contribution (>0) is whether they donate any positive amount.
- **Community Engagement.** The average of Savings Group, Listening Volunteer, and Listening Contribution.

Hypothesis. Correcting pessimistic beliefs about peers lowers the perceived social cost of engaging, so we expect community engagement to rise.

Own Health

- **Speaking to the Doctor (MH).** Whether the individual feels comfortable discussing mental health problems with a doctor. The exact wording asked whether they “feel comfortable discussing your mental health related problems with a doctor”.
- **Depression Scoring (Immediate).** Whether the individual agrees to take a depression screening (a PHQ-9 questionnaire) straight away, and Depression Scoring is whether they agree to take it either straight away or when called back later.
- **Listening to Helpline Numbers.** Whether the individual is willing to have a list of government mental health resources, namely local hospitals and facilities, read out to them.

Hypothesis. More comfort with community-based support could complement formal help-seeking (raising it) or substitute for it (lowering it).

Other Endline Outcomes

- **Memory (Numbers Remembered).** How many of five numbers, read out earlier in the survey, the individual correctly recalls, on a scale from zero to five.

Hypothesis. Presence of supportive peers can reduce stress and thereby reduce cognitive load (Mani et al., 2013; Lichand and Mani, 2020). If so, belief correction should reduce cognitive load and improve recall.

- **Physical Health Dialogue (with Family).** Whether the individual feels comfortable discussing physical health problems with friends and family. The exact wording asked whether they “feel comfortable discussing your physical health related problems with your friends and family”.

Hypothesis. Support on one dimension can lead to positive spillovers to other dimensions such as willingness to discuss physical health.

Demand for Information, Self-Efficacy, and Stigma

- **Information Session.** Whether the individual is willing to attend an NGO mental health information session. The exact wording was “We are thinking of organizing a 1-hour information session with the NGO and other participants to discuss mental health related concerns and give additional information. . . Would you like to participate?”
- **Listening to Good Practices.** Whether the individual is willing to hear suggestions on how to talk about mental health, such as how to start a conversation and become a better listener, with Immediate meaning they agree to hear them straight away rather than at a later time.

Hypothesis. Correcting pessimistic beliefs about peers lowers the perceived social cost of engaging, so we expect demand for information around these topics to rise.

- **Self Efficacy.** The average of three items in which the individual rated their confidence, on a scale from “not at all” to “almost certain”, about dealing with money and work, their child’s education, and starting a business. The exact wording of the three items was “How confident are you about your ability to deal with financial/employment related concerns in the coming months” (Goals (Finance)), “How confident are you that you will be able to arrange money for your child’s education” (Goals (Education)), and “how confident are you about your ability to start a small business?” (Goals (Business)).

Hypothesis. If individuals can rely on their network for support, correcting beliefs should raise financial self-efficacy.

- **Stigma (Information Session).** Whether the individual is willing to have their name shared with other community members as a session participant. The exact wording asked “Can we forward your name with other names of participants who said yes to others to encourage them?”
- **List Count.** The number of statements, out of four, that the individual agrees with in a list experiment. The mental health statement in the list was “People should remain friends with those suffering from mental health concerns”.
- **Depression Score Revelation.** Whether the individual is willing to share their total depression score with the surveyor.

Hypothesis. Because the treatment corrects beliefs about how community members support mental health, we expect at most a reduction in own stigma.

TABLE C.1. Effects on Additional Endline Outcomes

<i>Panel A: Effects on Demand for Information</i>				
	Information Session	Listening to Good Practices (Immediate)	Listening to Good Practices	
Treatment	0.0534 (0.0685)	-0.0507 (0.0846)	0.0263 (0.0636)	
<i>Bootstrap p-value</i>	0.430	0.511	0.697	
<i>q-values</i>	1	1	1	
Constant	0.674*** (0.0491)	0.551*** (0.0603)	0.821*** (0.0472)	
Observations	180	141	139	
R-squared	0.003	0.003	0.001	
<i>Panel B: Effects on Self Efficacy</i>				
VARIABLES	Goals (Finance)	Goals (Education)	Goals (Business)	Self Efficacy
Treatment	0.154 (0.264)	-0.200 (0.243)	0.0436 (0.278)	-0.0268 (0.222)
<i>Bootstrap p-value</i>	0.647	0.468	0.852	0.893
<i>q-values</i>	1	1	1	1
Constant	2.219*** (0.176)	2.729*** (0.183)	2.548*** (0.190)	2.481*** (0.151)
Observations	148	140	144	139
R-squared	0.002	0.005	0.000	0.000
<i>Panel C: Effects on Stigma</i>				
VARIABLES	Stigma (Information Session)	List Count	Depression Score Revelation	
Treatment	0.0313 (0.0430)	-0.0763 (0.211)	-0.0917 (0.0881)	
<i>Bootstrap p-value</i>	0.373	0.781	0.451	
<i>q-values</i>	1	1	1	
Constant	0.922*** (0.0338)	3.507*** (0.157)	0.925*** (0.0423)	
Observations	128	154	64	
R-squared	0.004	0.001	0.020	
Robust standard errors in parentheses. Wild bootstrap p-value reported, reps=999. *** p<0.01, ** p<0.05, * p<0.1				

Notes: Definitions of all outcomes are provided in Appendix Section C.1. We report robust standard errors. We also report wild bootstrap p values using the method outlined in Cameron et al. (2008) where we treat the NGO center as the cluster unit. The *q-values* (Benjamini et al., 2006) reported in each table treat the outcomes in the table as multiple hypotheses being tested together.

C.2. Heterogeneity.

TABLE C.2. Treatment Effects by Baseline Willingness to Talk about Mental Health

<i>Panel A: Effect on Engagement Outcomes</i>						
	Savings Group	Listening Volunteer	Listening Contribution	Contribution (in Rupees)	Contribution (>0)	Community Engagement
Treatment	0.374*** (0.137)	0.434*** (0.119)	0.204* (0.119)	11.86** (4.964)	7.817 (5.063)	0.315*** (0.104)
Willingness to talk (Mental Health)	0.466*** (0.122)	0.375*** (0.112)	0.186 (0.114)	10.19** (4.441)	6.190 (4.641)	0.295*** (0.0949)
Interaction	-0.265* (0.154)	-0.355** (0.151)	-0.106 (0.138)	-6.677 (6.223)	-5.553 (6.075)	-0.235** (0.117)
Constant	0.350*** (0.108)	0.280*** (0.0909)	0.625*** (0.100)	15.87*** (3.656)	26.07*** (4.083)	0.467*** (0.0876)
Observations	138	160	156	150	121	138
R-squared	0.185	0.103	0.053	0.069	0.029	0.152
Bootstrap p-value	0.0390	0.101	0.345	0.114	0.187	0.0561

<i>Panel B: Effect on Other Outcomes</i>			
VARIABLES	Information Session	Listening to Good Practices (Immediate)	Listening to Good Practices
Treatment	0.444*** (0.116)	0.0292 (0.152)	0.168 (0.129)
Willingness to talk (Mental Health)	0.488*** (0.105)	0.0389 (0.138)	0.182 (0.120)
Interaction	-0.510*** (0.142)	-0.144 (0.187)	-0.184 (0.150)
Constant	0.333*** (0.0918)	0.526*** (0.116)	0.684*** (0.108)
Observations	164	132	131
R-squared	0.143	0.010	0.027
Bootstrap p-value	0.0120	0.471	0.0340

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Note: This table shows heterogeneous effects on the demand for additional information by baseline willingness to engage with mental health related concerns. Definitions of all outcomes are provided in Appendix Section C.1. We report robust standard errors. We also report wild bootstrap p -values for the interaction term using the method outlined in Cameron et al. (2008) where we treat the NGO center as the cluster unit.

TABLE C.3. Heterogeneous Treatment Effects by Baseline Dialogue

<i>Panel A: Baseline Dialogue Around Mental Health</i>						
	Savings Group	Listening Volunteer	Listening Contribution	Contribution (in Rupees)	Contribution (>0)	Community Engagement
Treatment	0.0227 (0.0948)	0.0581 (0.107)	0.0808 (0.0651)	8.129* (4.226)	5.643 (3.770)	0.0437 (0.0665)
Baseline MH Dialogue (Below Median)	-0.305*** (0.107)	-0.195* (0.106)	-0.264*** (0.0943)	-12.53*** (3.957)	-6.565* (3.702)	-0.243*** (0.0753)
Interaction	0.282** (0.142)	0.206 (0.146)	0.119 (0.117)	-0.164 (5.632)	-2.510 (5.189)	0.217** (0.100)
Constant	0.805*** (0.0627)	0.636*** (0.0734)	0.864*** (0.0524)	28.90*** (2.951)	33.86*** (2.661)	0.772*** (0.0458)
Observations	150	174	170	163	129	150
R-squared	0.088	0.048	0.093	0.137	0.078	0.118
Bootstrap p-value	0.0931	0.185	0.296	0.979	0.456	0.108
<i>Panel B: Baseline Dialogue Around Financial Concerns</i>						
	Savings Group	Listening Volunteer	Listening Contribution	Contribution (in Rupees)	Contribution (>0)	Community Engagement
Treatment	0.0368 (0.0771)	-0.0950 (0.1000)	-0.0150 (0.0847)	0.887 (4.532)	1.296 (3.980)	0.00919 (0.0600)
Baseline FC Dialogue (Below Median)	-0.363*** (0.0974)	-0.453*** (0.0951)	-0.204** (0.0912)	-9.458** (4.170)	-4.641 (3.852)	-0.317*** (0.0673)
Interaction	0.183 (0.130)	0.436*** (0.138)	0.237** (0.119)	9.400 (5.948)	3.410 (5.408)	0.220** (0.0937)
Constant	0.875*** (0.0593)	0.806*** (0.0667)	0.857*** (0.0599)	28.44*** (3.246)	33.70*** (2.848)	0.844*** (0.0422)
Observations	150	174	170	163	129	150
R-squared	0.133	0.133	0.056	0.059	0.022	0.176
Bootstrap p-value	0.154	0.00100	0.0811	0.0280	0.312	0

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Note: This table shows heterogeneous effects on community engagement by a binary variable which indicates whether baseline frequency of dialogue with peers around mental health financial concerns respectively was less than/equal to the median. Definitions of all outcomes are provided in Appendix Section C.1. We report robust standard errors. We also report wild bootstrap p -values for the interaction term using the method outlined in Cameron et al. (2008) where we treat the NGO center as the cluster unit.

TABLE C.4. Heterogeneous Treatment Effects by Degree (Overall)

	(1) Savings Group	(2) Listening Volunteer	(3) Listening Contribution	(4) Contribution (in Rupees)	(5) Contribution (>0)	(6) Community Engagement
Treatment	0.0117 (0.110)	0.159 (0.107)	0.150* (0.0901)	2.992 (4.303)	-3.434 (3.632)	0.117 (0.0774)
Degree (Overall)	-0.00736 (0.0137)	0.0104 (0.0126)	0.00258 (0.0128)	-0.464 (0.500)	-0.893* (0.464)	0.00156 (0.00846)
Interaction	0.0307* (0.0183)	-0.00116 (0.0172)	-0.00625 (0.0160)	0.838 (0.733)	1.532** (0.632)	0.00432 (0.0123)
Constant	0.696*** (0.0755)	0.499*** (0.0746)	0.728*** (0.0707)	24.59*** (3.052)	34.88*** (2.466)	0.656*** (0.0572)
Observations	150	174	170	163	129	150
R-squared	0.052	0.034	0.024	0.038	0.050	0.049
Bootstrap p-value	0.0450	0.910	0.566	0.153	0.0651	0.633

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Note: This table shows heterogeneous effects on various measures of community engagement by the number of connections in the overall network. Definitions of all outcomes are provided in Appendix Section C.1. We report robust standard errors. We also report wild bootstrap p -values for the interaction term using the method outlined in Cameron et al. (2008) where we treat the NGO center as the cluster unit.

TABLE C.5. Treatment Effects by Stigma (Mental Health)

	(1) Savings Group	(2) Listening Volunteer	(3) Listening Contribution	(4) Contribution (in Rupees)	(5) Contribution (>0)	(6) Community Engagement
Treatment	0.202** (0.0982)	0.146 (0.105)	0.125 (0.0866)	3.224 (4.171)	-0.930 (3.726)	0.149** (0.0703)
Stigma (Mental Health)	0.110 (0.102)	-0.148 (0.106)	-0.0453 (0.0884)	-7.107* (4.262)	-7.071* (4.033)	-0.0175 (0.0732)
Interaction	-0.0907 (0.144)	0.0921 (0.150)	-0.00473 (0.124)	7.291 (6.001)	9.349* (5.470)	-0.0119 (0.103)
Constant	0.632*** (0.0685)	0.610*** (0.0742)	0.775*** (0.0613)	26.25*** (2.912)	33.87*** (2.695)	0.684*** (0.0491)
Observations	139	161	157	151	122	139
R-squared	0.045	0.053	0.028	0.050	0.040	0.056
Bootstrap p-value	0.527	0.451	0.951	0.274	0.00300	0.854

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Note: This table shows heterogeneous effects on various measures of community engagement by whether the individual has stigma against mental health during the baseline in that they believe that “people should stay away from those with mental health issues”. Definitions of all outcomes are provided in Appendix Section C.1. We report robust standard errors. We also report wild bootstrap p -values for the interaction term using the method outlined in Cameron et al. (2008) where we treat the NGO center as the cluster unit.

TABLE C.6. Treatment Effects by Underestimators

<i>Panel A: Underestimation of Willingness to Engage around Mental Health</i>						
	Savings Group	Listening Volunteer	Listening Contribution	Contribution (in Rupees)	Contribution (>0)	Community Engagement
Treatment	0.0804 (0.116)	0.0833 (0.130)	0.117 (0.127)	11* (5.723)	8.636 (5.587)	0.110 (0.0759)
Underestimator (MH)	-0.131 (0.122)	-0.287** (0.130)	0.0192 (0.125)	5.667 (5.316)	6.329 (5.263)	-0.104 (0.0820)
Interaction	0.0799 (0.149)	0.128 (0.163)	0.00541 (0.147)	-6.310 (6.923)	-7.626 (6.588)	0.0215 (0.101)
Constant	0.813*** (0.0992)	0.750*** (0.110)	0.750*** (0.110)	19.33*** (4.610)	26.36*** (4.724)	0.771*** (0.0650)
Observations	126	146	144	138	113	126
R-squared	0.045	0.091	0.024	0.036	0.024	0.080
Bootstrap p-value	0.632	0.429	0.974	0.402	0.224	0.848
<i>Panel B: Underestimation of Willingness to Engage around Financial Concerns</i>						
	Savings Group	Listening Volunteer	Listening Contribution	Contribution (in Rupees)	Contribution (>0)	Community Engagement
Treatment	0.107 (0.122)	0.200 (0.135)	0.100 (0.114)	7.347 (5.352)	5.196 (4.983)	0.118 (0.0874)
Underestimator (FC)	-0.0295 (0.126)	0.0294 (0.134)	0 (0.117)	1.562 (5.032)	1.786 (4.698)	-0.00128 (0.0857)
Interaction	0.0387 (0.154)	0.0206 (0.169)	0.0389 (0.141)	-0.694 (6.706)	-2.689 (6.161)	0.0325 (0.109)
Constant	0.737*** (0.103)	0.500*** (0.113)	0.750*** (0.0982)	21.84*** (4.209)	29.64*** (3.967)	0.684*** (0.0689)
Observations	126	147	144	138	111	126
R-squared	0.026	0.045	0.024	0.032	0.015	0.053
Bootstrap p-value	0.824	0.838	0.851	0.952	0.705	0.737

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Note: This table shows heterogeneous effects on community engagement by whether the individual underestimates community willingness to engage around mental health (MH) and financial concerns (FC), respectively. Definitions of all outcomes are provided in Appendix Section C.1. We report robust standard errors. We also report wild bootstrap p -values for the interaction term using the method outlined in Cameron et al. (2008) where we treat the NGO center as the cluster unit.

TABLE C.7. Heterogeneous Treatment Effects by Baseline Beliefs about Stress

<i>Panel A: Underestimation of Stress in the Community</i>						
	Savings Group	Listening Volunteer	Listening Contribution	Contribution (in Rupees)	Contribution (>0)	Community Engagement
Treatment	0.102 (0.0958)	0.192* (0.102)	0.0901 (0.0899)	4.487 (4.328)	1.750 (3.940)	0.110 (0.0706)
Underestimator (Stress)	-0.0210 (0.116)	-0.00905 (0.119)	0.0444 (0.104)	0.590 (4.609)	-1.700 (4.214)	-0.00560 (0.0814)
Interaction	0.0534 (0.148)	-0.0134 (0.164)	0.0870 (0.124)	5.146 (6.308)	2.677 (5.773)	0.0400 (0.102)
Constant	0.735*** (0.0769)	0.538*** (0.0809)	0.737*** (0.0724)	23.47*** (3.274)	32.50*** (2.998)	0.696*** (0.0534)
Observations	128	149	146	140	113	128
R-squared	0.024	0.039	0.035	0.036	0.011	0.046
Bootstrap p-value	0.550	0.940	0.290	0.436	0.607	0.541
<i>Panel B: Beliefs about Stress in the Community</i>						
	Savings Group	Listening Volunteer	Listening Contribution	Contribution (in Rupees)	Contribution (>0)	Community Engagement
Treatment	0.143 (0.131)	0.200 (0.140)	0.140 (0.104)	7.454 (5.039)	3.895 (4.644)	0.141 (0.0891)
Stress Belief (>4)	0.0965 (0.120)	0.0837 (0.122)	-0.0192 (0.107)	2.335 (4.668)	4.387 (4.312)	0.0665 (0.0839)
Interaction	-0.0393 (0.157)	-0.0248 (0.170)	-0.0380 (0.133)	-2.293 (6.484)	-1.933 (5.881)	-0.0305 (0.110)
Constant	0.667*** (0.0978)	0.481*** (0.0975)	0.769*** (0.0838)	22.31*** (3.568)	29*** (3.407)	0.653*** (0.0676)
Observations	128	149	146	140	113	128
R-squared	0.031	0.044	0.023	0.027	0.022	0.051
Bootstrap p-value	0.679	0.871	0.758	0.784	0.770	0.721
Robust standard errors in parentheses						
*** p<0.01, ** p<0.05, * p<0.1						

Note: Panel A shows heterogeneous effects on various measures of community engagement by whether the individual underestimates the level of stress in their community, i.e., their belief is less than the true community-level stress. Panel B shows heterogeneous effects on various measures of community engagement by whether the individual's belief about the level of stress in their community is above the proportion who are actually not willing to engage (i.e., 40%). This is to check if those who think more than 40% are stressed and might assume that these individuals are not willing to engage change their behavior when they are told that majority are willing to engage. Definitions of all outcomes are provided in Appendix Section C.1. We report robust standard errors. We also report wild bootstrap p -values for the interaction term using the method outlined in Cameron et al. (2008) where we treat the NGO center as the cluster unit.

D. Mechanisms.

TABLE D.1. List Experiment to detect Social Desirability Concerns

	3 statements + Statement about engagement with MH	3 statements	Average Difference between 4 and 3 statements
Mean Agreements	2.971	2.217	0.754***
Observations	670		

	3 statements + 2 statements about engagement with MH and FC	3 statements	Average Difference between 5 and 3 statements
Mean Agreements	3.717	2.217	1.499***
Observations	504		

Notes: These tables present the results of the list experiments where individuals were randomly divided into groups and either asked how many of the three statements they agree with (i.e., control) or asked how many of 4 and 5 statements they agree with (i.e., treated). The additional statements were relating to engagement with mental health and financial concerns respectively. "MH" refers to mental health and "FC" refers to financial concerns. The first two columns show the mean number of agreements when different number of statements are provided. The third column computes the difference between treated and control conditions.

TABLE D.2. Additional Experiment to detect Social Desirability Concerns

VARIABLES	(1) Savings Group	(2) Listening Service	(3) Savings Group	(4) Listening Service
Increased Distance between Enumerator and Respondent	0.00398 (0.0280)	0.0326 (0.0324)	0.0241 (0.0365)	0.0497 (0.0453)
Treatment			-0.0609 (0.0397)	-0.0305 (0.0467)
Increased Distance x Treated			-0.0352 (0.0555)	-0.0311 (0.0647)
Constant	0.807*** (0.0199)	0.690*** (0.0233)	0.838*** (0.0263)	0.706*** (0.0326)
Observations	791	791	791	791
R-squared	0.000	0.001	0.011	0.004

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Notes: This table presents the results of an experiment where we randomly vary whether the enumerator enters the respondent's answer (*Increased Distance*=0) or gives them the device to answer it privately (*Increased Distance*=1). We also interact it with the treatment to see if the response being visible to the enumerator affects responses differently across the two groups. We report robust standard errors.

E. Follow-up Survey.

E.1. Follow-up Survey Outcomes.

The follow-up survey outcomes are listed below.

- (1) Well-being: Happiness, Life-Satisfaction, Stress
- (2) Self-reported Dialogue
 - (a) Indicator whether they have initiated a conversation with their peers to discuss matters relating to either of their mental and financial concerns.
 - (b) Number of peers they have initiated conversations with to discuss matters relating to either of their mental and financial concerns.
 - (c) Indicator whether their peers have initiated a conversation with them to discuss matters relating to their own mental and financial concerns.
- (3) Physical Health Safety
 - (a) How often did they follow COVID rules (distancing, wearing masks)?
 - (b) How often did they wear safety gear (masks, gloves, jacket) while working in hazardous conditions?
- (4) Altruism towards community members (Dictator Game)
- (5) Self-reported contact with a doctor/helpline for mental or physical health concerns

E.2. Follow-up Survey: Main Results. The results on network interactions have been presented in the main text in Table 5. Next, we find in Table E.1 that individuals in the treatment group are likely to donate Rs. 31 more than the control group in a hypothetical dictator game where they can split Rs. 200 with a randomly chosen member of their community. This result is robust to the inclusion of unbalanced controls as well. However, we also document that individuals in the treatment group report lower happiness, higher stress, and lower life satisfaction. At the same time, we also find that the treatment group wears work safety gear significantly more often than the control group, conditional on having worked. Given that we find an (a) increase in dialogue around mental health and (b) investments in self-care (in terms of greater adoption of safety gear), a potential mechanism explaining lower well-being could be that the role of dialogue in alleviating stress becomes ineffective in the presence of correlated risks such as COVID-19.

However, in the absence of real-time data on the location of these individuals, we acknowledge that we cannot rule out the possibilities that (a) the treatment group has lower well-being because they have migrated to an area of low COVID risk and hear about the impact of the pandemic on their peers via increased dialogue or (b) the treatment group is located in an area of high COVID risk and well-being is consequently lower. We have checked balance on several baseline variables and do not ex ante see any reason why the treatment group will be more or less likely to migrate. However, if this were to be the case, we believe that mechanism (a) seems to be more likely. This is because the result in Table E.1 shows that the treatment group wears COVID-19 masks less often than the control group. 92% of our sample reports to have *always* worn a mask in the past two weeks before the call, and the negative effect on the treatment is only driven by the remaining 8%. Individuals in low-risk areas may have been less likely to wear COVID-19 safety masks. Additionally, the proportion of individuals for whom the question on safety gear adoption is missing is higher for the treatment group after controlling for unbalanced baseline variables. This may imply that they are away from their place of work.

TABLE E.1. Other Outcomes

<i>Panel A: Effects on Other Outcomes</i>				
VARIABLES	Masks (How often)	Safety Gear (How often)	Maintain Distance (How often)	Mental Health- Doctor (Made Call)
Treatment	-0.179 (0.108)	0.783*** (0.223)	-0.0351 (0.145)	-0.0229 (0.0534)
<i>q-values</i>	0.024	0.008	0.19	0.14
Constant	3.943*** (0.0419)	2.977*** (0.180)	3.604*** (0.0907)	0.0943** (0.0405)
Observations	104	69	104	109
R-squared	0.027	0.120	0.001	0.002

<i>Panel B: Effects on Well-being</i>				
VARIABLES	Happiness	Life Satisfaction	Stress	Altruism (Dictator Game)
Treatment	-0.568*** (0.176)	-0.434** (0.168)	0.240 (0.167)	30.66** (13.68)
<i>q-values</i>	0.024	0.024	0.054	0.048
Constant	3.208*** (0.115)	3.434*** (0.109)	2.684*** (0.125)	54.90*** (9.793)
Observations	103	103	102	104
R-squared	0.094	0.062	0.020	0.047

Robust standard errors in parentheses
*** p<0.01, ** p<0.05, * p<0.1

Notes: The *q-values* (Benjamini et al., 2006) reported in each table treat all the outcomes in this table as multiple hypotheses being tested together. Standard errors are robust.

Thus, while we document positive effects on dialogue, altruism, and safety gear adoption, we also document negative effects on happiness and life satisfaction, and a positive effect on stress. These results highlight that the correlated nature of risk can affect the success of belief-shifting interventions.

F. Larger Scale Experiment.

We conducted a demographic survey with a sample of ~ 800 individuals in 2 NGO centers in Delhi in 2023 to alleviate the concern that the baseline patterns are specific to the sub-sample or because the survey was conducted during the pandemic. Table F.1 shows that the baseline patterns of low engagement both in terms of low dialogue and gaps in the number of connections in advice networks and overall networks continue to hold. The proportion of individuals who have never or rarely had a conversation about mental health and financial concerns is lower than before but still as high as 30%. We also separately measured the number of links for financial and mental health-related advice-taking and find that the average number of links in the former is even lower than that of advice-taking around mental health. We find that those who are more optimistic about their community’s willingness to engage are more likely to have fewer gaps in connections between overall and financial networks. We also continue to find that individuals report a high willingness to engage and an even larger proportion underestimate how willing their community is to engage with them. This reassures us that the baseline patterns measured before the experiment continue to hold.

Next, we present the results from the implementation of a similar information treatment. Half of these individuals were chosen to be treated and given the same information as the treated individuals in the RCT with two crucial differences that make this a weaker replication of the original experiment. First, we provided individuals with information about the average willingness

to engage in 2021. They were told that this is not information from 2023 and were asked to answer questions assuming this were true today. Further, unlike the original treatment where individuals were told that the information we are providing is from their own community, in the additional sample, we told individuals that the information is collected from other similar communities of waste-pickers managed by the NGO in Delhi. This is because this center was not a part of our original sample and even if it were, the center composition would have changed due to COVID-induced migration.

Table F.2 shows the balance checks for this sample. We find that while there is balance in most variables, treatment and control groups are not similar in terms of the number of overall network connections, willingness to engage in mental health related dialogue, and the proportion whose belief about community’s willingness to engage is lower than the delivered information with treated participants being better off along most of these variables. We use post double selection Lasso (Belloni et al., 2014) to estimate the treatment effects while accounting for this imbalance. The effect on willingness to participate in savings groups and listening services is shown in Tables F.4 and F.6. We continue to find evidence that beliefs about community willingness to engage significantly affect network engagement. We also present robustness checks where we exclude those who were a part of the previous study or report having heard about their community’s views on engagement via their peers in Tables F.5 and F.7. This is to ensure that the previous information delivery does not affect our findings. We find that the results are similar in direction, magnitude, and statistical significance whether these individuals are included or not.

However, the mechanisms behind this treatment are different from the original treatment in that individuals are provided information about other communities like theirs from two years ago. As a result, we find negative average effects on willingness to participate in savings groups. Disentangling this further, we find that those who are pessimistic about their community, are significantly less likely to engage when they are told that other communities are more willing to engage. Those who are optimistic about the community behave in the opposite manner. This pattern also holds for listening services even though the negative average effect is not significant. These effects can arise if those who believe that their community is not very willing to engage do not treat the new information as a positive signal about their own community but instead compare their community with other seemingly more supportive communities and draw a negative inference. This highlights the importance of the type of reference group whose beliefs are conveyed, as beliefs about peers can be updated either directly or through relative comparisons.

Regardless of how the information affects individual engagement, these results confirm that information about own community’s willingness to engage (relative to others, in this case) can affect own willingness to engage with the network. Not only this, despite the weaker treatment, as Table F.8 shows, we detect a large increase in financial contributions made by the participants for setting up savings groups and listening services. These contributions are significantly higher than the control group for treated participants who have lower initial predictions about their community’s willingness to engage with them, i.e., those who think that not many people from their community would be willing to engage with them.⁴⁴ This suggests that while they may be less willing to engage upon hearing the information, they are more willing to finance avenues for such interactions to be set up in future.

⁴⁴Note that this outcome is only accurately collected for the second center which is why the subsample here is close to half of the entire sample.

F.1. Results from the Larger Scale Experiment.

TABLE F.1. Summary Statistics

	(1)	
Age	34.03	(12.35)
Female	0.549	(0.498)
Income	6780.9	(3142.1)
Degree (Overall)	4.655	(2.077)
Degree (MH Advice)	2.431	(1.310)
Degree (FC Advice)	2.216	(1.324)
Happiness (1-5)	3.326	(1.140)
Willingness to Talk (FC)	0.898	(0.303)
Willingness to Talk (MH)	0.880	(0.325)
Beliefs (MH; 0-10)	3.753	(1.800)
Beliefs (FC; 0-10)	3.571	(1.864)
Dialogue (MH; 1-4)	2.930	(1.011)
MH talk (Never)	0.113	(0.316)
MH talk (Rarely)	0.210	(0.407)
MH talk (Sometimes)	0.312	(0.464)
MH talk (Often)	0.365	(0.482)
Dialogue (FC; 1-4)	2.889	(1.010)
FC talk (Never)	0.119	(0.324)
FC talk (Rarely)	0.215	(0.411)
FC talk (Sometimes)	0.325	(0.469)
FC talk (Often)	0.341	(0.474)
Observations	791	

mean coefficients; sd in parentheses

* $p < 0.05$, ** $p < 0.01$, *** $p < 0.001$

Notes: The above table shows the summary statistics (mean and standard deviation) for various demographic characteristics of interest for the additional sample in 2023. "MH" refers to mental health concerns and "FC" refers to financial concerns.

TABLE F.2. Balance Tests for the Full Sample in 2023

	(1)	(2)	(3)
	Control	Treatment	(1) vs. (2), p-value
Female	0.558	0.539	0.591
Age	33.901	34.144	0.783
Income	6911.429	6657.143	0.256
Degree (FC Advice)	2.275	2.160	0.222
Degree (MH Advice)	2.410	2.451	0.666
Degree (Overall)	4.491	4.810	0.031
Dialogue (MH; 1-4)	2.919	2.941	0.767
Dialogue (FC; 1-4)	2.894	2.884	0.898
Willingness to Talk (MH)	0.857	0.901	0.056
Willingness to Talk (FC)	0.881	0.914	0.125
Beliefs (MH; 0-10)	3.678	3.825	0.251
Beliefs (FC; 0-10)	3.564	3.579	0.909
Underestimators (MH)	0.823	0.771	0.067
<i>N</i>	385	406	

Notes: This table presents the results of balance tests between the treatment and control group for the entire replication sample. The first two columns show the means of the variables for the Control and Treatment Group and the third column shows the p values for the two-sided test that the two means are equal. "MH" refers to mental health concerns and "FC" refers to financial concerns. Underestimators is a binary variable equal to 1 if the individual's belief about proportion of community willing to engage is less than six, i.e., the information provided to the treatment group. *Degree* refers to the number of connections

TABLE F.3. Balance Tests for Center 2 in 2023

	(1)	(2)	(3)
	Control	Treatment	(1) vs. (2), p-value
Female	0.437	0.427	0.856
Age	30.934	29.653	0.266
Income	8166.887	8370.000	0.593
Degree (FC Advice)	2.318	2.153	0.308
Degree (MH Advice)	2.616	2.567	0.757
Degree (Overall)	4.954	4.767	0.478
Dialogue (MH; 1-4)	2.722	2.693	0.807
Dialogue (FC; 1-4)	2.755	2.660	0.419
Willingness to Talk (MH)	0.874	0.927	0.129
Willingness to Talk (FC)	0.914	0.927	0.684
Beliefs (MH; 0-10)	3.954	3.960	0.975
Beliefs (FC; 0-10)	3.411	3.547	0.527
Underestimator (MH)	0.755	0.733	0.668
N	156	156	

Notes: This table presents the results of balance tests between the treatment and control group for the second center in the replication sample. We present these balance tests separately as the mental health fund outcome was only accurately measured for this center. The first two columns show the means of the variables for the Control and Treatment Group and the third column shows the p values for the two-sided test that the two means are equal. "MH" refers to mental health concerns and "FC" refers to financial concerns. Underestimators is a binary variable equal to 1 if the individual's belief about proportion of community willing to engage is less than six, i.e., the information provided to the treatment group. *Degree* refers to the number of connections

TABLE F.4. Replication Exercise: Effect on Willingness to Participate in Savings Groups

VARIABLES	(1) Savings Group	(2) Savings Group	(3) Savings Group
Treatment	-0.0693** (0.0270)	-0.0726*** (0.0272)	-0.0712*** (0.0264)
Beliefs (MH)		-0.0311 (0.0221)	
Treatment x Beliefs (MH)		-0.0107 (0.0247)	
Beliefs (FC)			0.0616*** (0.0175)
Treatment x Beliefs (FC)			0.0592** (0.0266)
Constant	0.882*** (0.0334)	0.976*** (0.0545)	0.891*** (0.0451)
Observations	789	789	789
Number of groups	0	0	0

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Notes: This table shows regression results from the replication sample in 2023. We use post double selection Lasso (Belloni et al., 2014) with robust standard errors to estimate the treatment effects where we select from a range of baseline variables such as gender, age, income, number of network connections, dialogue intensity, and willingness to engage. Column 1 shows the main treatment effects while Columns 2 and 3 interact the treatment indicator with beliefs. "Beliefs" refers to the respondent's prediction (standardized) about how many community members would be willing to engage around mental or financial concerns. "MH" refers to mental health concerns and "FC" refers to financial concerns.

TABLE F.5. Replication Exercise: Effect on Willingness to Participate in Savings Groups (Robust)

VARIABLES	(1) Savings Group	(2) Savings Group	(3) Savings Group
Treatment	-0.0759** (0.0297)	-0.0748** (0.0297)	-0.0806*** (0.0290)
Beliefs (MH)		-0.0400 (0.0255)	
Treatment x Beliefs (MH)		-0.00215 (0.0277)	
Beliefs (FC)			0.0709*** (0.0195)
Treatment x Beliefs (FC)			0.0573** (0.0289)
Constant	0.944*** (0.0327)	0.968*** (0.0586)	0.950*** (0.0443)
Observations	696	696	696
Number of groups	0	0	0

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Notes: This table shows regression results from the replication sample in 2023, excluding those who were a part of the previous study or report having heard about their community's views on engagement via their peers. We use post double selection Lasso (Belloni et al., 2014) with robust standard errors to estimate the treatment effects where we select from a range of baseline variables such as gender, age, income, number of network connections, dialogue intensity, and willingness to engage. Column 1 shows the main treatment effects while Columns 2 and 3 interact the treatment indicator with beliefs. "Beliefs" refers to the respondent's prediction (standardized) about how many community members would be willing to engage around mental or financial concerns. "MH" refers to mental health concerns and "FC" refers to financial concerns.

TABLE F.6. Replication Exercise: Effect on Willingness to Participate in Listening Service

VARIABLES	(1) Listening Service	(2) Listening Service	(3) Listening Service
Treatment	-0.0442 (0.0323)	-0.0444 (0.0324)	-0.0430 (0.0319)
Beliefs (MH)		-0.0677*** (0.0253)	
Treatment x Beliefs (MH)		0.0287 (0.0304)	
Beliefs (FC)			0.0213 (0.0230)
Treatment x Beliefs (FC)			0.0953*** (0.0324)
Constant	0.730*** (0.0226)	0.789*** (0.0609)	0.695*** (0.0523)
Observations	789	789	789
Number of groups	0	0	0

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Notes: This table shows regression results from the replication sample in 2023. We use post double selection Lasso (Belloni et al., 2014) with robust standard errors to estimate the treatment effects where we select from a range of baseline variables such as gender, age, income, number of network connections, dialogue intensity, and willingness to engage. Column 1 shows the main treatment effects while Columns 2 and 3 interact the treatment indicator with beliefs. "Beliefs" refers to the respondent's prediction (standardized) about how many community members would be willing to engage around mental or financial concerns. "MH" refers to mental health concerns and "FC" refers to financial concerns.

TABLE F.7. Replication Exercise: Effect on Willingness to Participate in Listening Service (Robust)

VARIABLES	(1) Listening Service	(2) Listening Service	(3) Listening Service
Treatment	-0.0290 (0.0345)	-0.0270 (0.0345)	-0.0316 (0.0342)
Beliefs (MH)		-0.0747*** (0.0283)	
Treatment x Beliefs (MH)		0.0241 (0.0326)	
Beliefs (FC)			0.0250 (0.0254)
Treatment x Beliefs (FC)			0.0957*** (0.0348)
Constant	0.721*** (0.0248)	0.818*** (0.0673)	0.718*** (0.0437)
Observations	696	696	696
Number of groups	0	0	0

Robust standard errors in parentheses

*** p<0.01, ** p<0.05, * p<0.1

Notes: This table shows regression results from the replication sample in 2023, excluding those who were a part of the previous study or report having heard about their community's views on engagement via their peers. We use post double selection Lasso (Belloni et al., 2014) with robust standard errors to estimate the treatment effects where we select from a range of baseline variables such as gender, age, income, number of network connections, dialogue intensity, and willingness to engage. Column 1 shows the main treatment effects while Columns 2 and 3 interact the treatment indicator with beliefs. "Beliefs" refers to the respondent's prediction (standardized) about how many community members would be willing to engage around mental or financial concerns. "MH" refers to mental health concerns and "FC" refers to financial concerns.

TABLE F.8. Replication Exercise: Effect on Contribution to Set up Savings Groups and Listening Services

VARIABLES	(1) Contribution (in Rupees.)	(2) Contribution (in Rupees.)	(3) Contribution (in Rupees.)	(4) Contribution (in Rupees.)
Treatment	2.468 (2.299)	2.916 (2.415)	2.334 (2.272)	2.632 (2.330)
Beliefs (MH)		1.020 (1.281)		
Treatment x MH Beliefs		-4.470* (2.520)		
Beliefs (FC)			0.414 (0.901)	
Treatment x FC Beliefs			-3.409** (1.698)	
Beliefs (Average)				1.030 (1.349)
Treatment x Avg. Beliefs				-5.158** (2.456)
Constant	16.96*** (1.364)	16.84*** (1.359)	17.00*** (1.362)	16.95*** (1.366)
Observations	312	312	312	312
R-squared	0.004	0.019	0.014	0.023

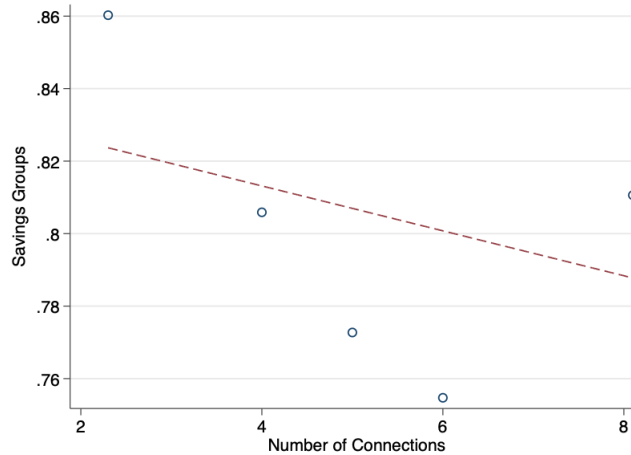
Robust standard errors in parentheses
*** p<0.01, ** p<0.05, * p<0.1

Notes: This table shows regression results from one center in the replication sample in 2023 for which this outcome was accurately measured, with robust standard errors. Column 1 shows the main treatment effects while Columns 2, 3, and 4 interact the treatment indicator with individual beliefs. "Beliefs" refers to the respondent's prediction (standardized) about how many community members would be willing to engage around mental or financial concerns. "MH" refers to mental health concerns and "FC" refers to financial concerns, and "Average" is the average of the two.

G. Theory.

G.1. Evidence for Model Assumptions. The following figures show correlations between degree centrality and survey measures of engagement with the network.

FIGURE G.1. Correlation between Degree and Willingness to Participate in Savings Groups



Notes: The figure shows the correlation between willingness to participate in savings group and the degree centrality of the agent using a binscatter with 5 quantiles.

G.2. Proof for Lemma 1.

Consider the distribution function of the probability mass function $\tilde{P}(d)$

$$\tilde{\mathbf{P}}(\bar{\mathbf{d}}) = \sum_{d=1}^{d=\bar{d}} \tilde{P}(d)$$

We first prove that the CDF corresponding to P is first order stochastically dominated by the CDF corresponding to $\tilde{P}$. Consider $\bar{d} \leq E[d]$. In this case, $\sum_{d=1}^{d=\bar{d}} \tilde{P}(d) = \sum_{d=1}^{d=\bar{d}} \frac{d}{E[d]} P(d) \leq \sum_{d=1}^{d=\bar{d}} P(d)$ since $\frac{d}{E[d]} \leq 1$.

When $\bar{d} > E[d]$, then $\sum_{d=1}^{d=\bar{d}} \tilde{P}(d) = 1 - \sum_{d=\bar{d}}^{d=d_{max}} \tilde{P}(d)$. The second term can be written as $\sum_{d=\bar{d}}^{d=d_{max}} \tilde{P}(d) = \sum_{d=\bar{d}}^{d=d_{max}} \frac{d}{E[d]} P(d) > \sum_{d=\bar{d}}^{d=d_{max}} P(d)$ since $\frac{d}{E[d]} > 1$. So, $\sum_{d=1}^{d=\bar{d}} \tilde{P}(d) < \sum_{d=1}^{d=\bar{d}} P(d)$ in this case as well.

Hence, $\tilde{\mathbf{P}}(\bar{\mathbf{d}})$ first order stochastically dominates $\mathbf{P}(\bar{\mathbf{d}})$.

We can rewrite

$$\sum_d \tilde{P}(d) H(c(d, a)) = H(c(d_{max}, a)) - \sum_{d=1}^{d=d_{max}-1} \tilde{\mathbf{P}}(\mathbf{d}) (H(c(d+1, a)) - H(c(d, a)))$$

and similarly rewrite

$$\sum_d P(d) H(c(d, a)) = H(c(d_{max}, a)) - \sum_{d=1}^{d=d_{max}-1} \mathbf{P}(\mathbf{d}) (H(c(d+1, a)) - H(c(d, a)))$$

Given that H is weakly increasing in degree d and $\tilde{\mathbf{P}}(\mathbf{d}) < \mathbf{P}(\mathbf{d})$, this implies that-

$$\sum_d \tilde{P}(d) H(c(d, a)) \geq \sum_d P(d) H(c(d, a))$$

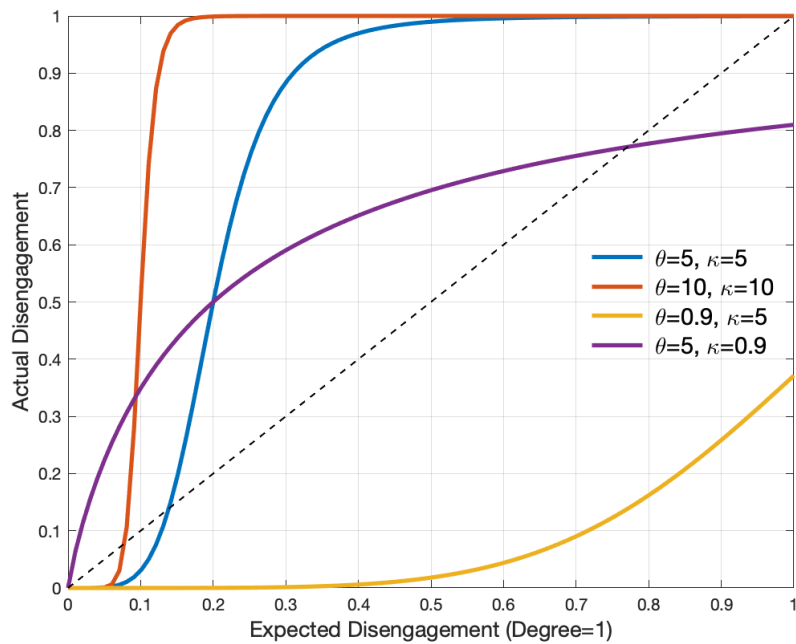
G.3. Proof for Proposition 1.

If c and consequently H are strictly increasing in degree, Lemma 1 implies that $\tilde{E}[H(c(d, z'))] > E[H(c(d, z'))] \forall z' \in (0, 1]$. This means that for any fixed point z that satisfies Equation 2, $\tilde{E}[z] > z$.

Given that we have assumed H to be such that there are three equilibria, of which one is unstable, this implies that (a) the fixed points a_l and a_h solving Equation 2 will not solve Equation 3, and (b) the fixed points $\tilde{a}_h$ and $\tilde{a}_l$ that solve Equation 3 will satisfy $\tilde{a}_l < a_l$ and $\tilde{a}_h > a_h$.

H. Structural Estimation.

FIGURE H.1. Different shapes of $H(\theta, \kappa)$ for various values of θ and κ



Notes: The figure plots the benefit distribution H distributed log-logistic with shape parameter κ and strategic complementarity parameter θ . Degree is normalized to be 1 in this example.

TABLE H.1. Structural Estimation Prediction Checks for various measures of Engagement

Variable	Objective Function	Predicted Mean (Treated)	Actual Mean (Treated)	Predicted Mean (Control)	Actual Mean (Control)
Listening Service	0.0005	0.309	0.299	0.440	0.460
Savings Groups	0.0012	0.217	0.187	0.318	0.333
Community Engagement	0.0001	0.201	0.200	0.332	0.338

Notes: This table shows the model fit for the estimated values of θ^* and κ^* for three different choice of outcomes. The objective function is computed at the estimated parameters and is equal to the sum of the squared gap between the mean of the treatment and control groups predicted by the model and in the data respectively.

H.1. Constructing Confidence Sets using Quasi MCMC Methods.

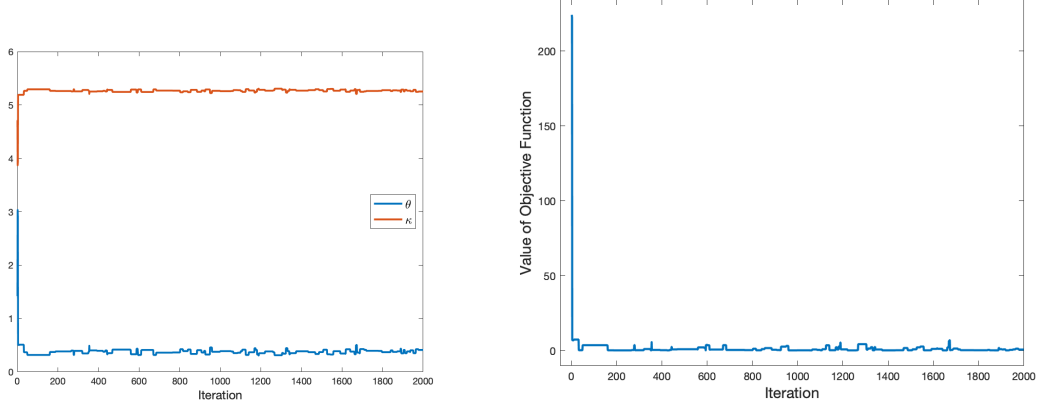
We estimate the confidence intervals for θ and κ as follows. We first compute the covariance matrix for the moment conditions (i.e., the mean of the treatment group and the control group) using the variance-covariance matrix returned via the OLS regression on treatment and control indicators and no constant. We denote this covariance matrix as $c\hat{ov}$. We redefine our weighted loss function in terms of $c\hat{ov}$ by placing weights on the moments using the inverse of the covariance matrix. Next, we compute the loss function for a perturbed value of θ and κ around θ^* and κ^* computed via pattern-search. We use a step-size of 0.001 and compute a 2x2 Jacobian matrix indicating how much the two moment conditions change due to a change in each parameter. This Jacobian matrix J is used to construct another covariance matrix cov_{shock} as follows: $cov_{shock} = \text{inv}(J' \text{inv}(c\hat{ov}) J)$.

Now, we proceed with the MCMC chain. We first select an initial value of θ_0 and κ_0 that is equal to θ^* and κ^* plus an additional shock coming from a multivariate normal distribution whose variance-covariance matrix is specified by cov_{shock} . We compute the weighted loss function for this chosen value of θ and κ . Then, select another value θ_1 and κ_1 which is equal to θ and κ plus another shock. We compute the loss function again. Now, we choose the value θ_1 and κ_1 if the loss function is lower than the previous one. If not, the new θ_1 and κ_1 are still chosen with a probability α_1 where α_t is specified as follows:

$$\alpha_t = e^{-\frac{1}{2}(Loss_t - Loss_{t-1})}$$

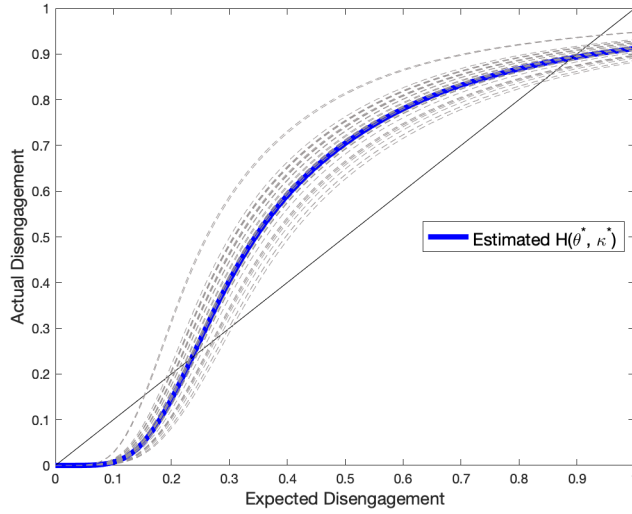
Crucially, α_t is close to zero if $Loss_t$ is very high compared to $Loss_{t-1}$ but is positive if the difference is not very large. If θ_1 and κ_1 are selected, then θ_2 and κ_2 are constructed as before by adding a shock to θ_1 and κ_1 . If not, θ_2 and κ_2 are constructed by adding a shock to θ_0 and κ_0 . The process continues until the chain converges in distribution. Once the chain converges, we draw θ and κ from the converged distribution to construct the confidence set. We find that the chain converges quickly. This is shown in Figure H.2, which plots the chosen values in each iteration and the loss associated with each value. We draw θ and κ from the last 500 iterations and plot the best response curve for these values in Figure H.3.

FIGURE H.2. Convergence of θ and κ over 2000 iterations



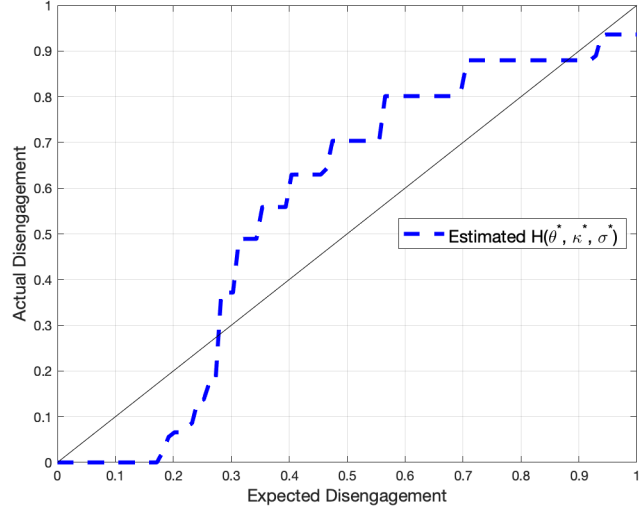
Notes: The figure on the left plots θ and κ over various iterations. In each iteration t , a candidate value of θ and κ is chosen with probability 1 if $Loss_t$ is less than $Loss_{t-1}$ and with a probability $e^{-0.5(Loss_t - Loss_{t-1})}$ if it is higher than before. The figure on the right plots the objective function over various iterations.

FIGURE H.3. Actual and Estimated Disengagement (Confidence Set)



Notes: The figure plots actual disengagement a and expected disengagement $\sum_d \tilde{P}(d)(H(c(d, a)))$ at the estimated θ^*, κ^* (in blue) and at various θ and κ pairs computed using Quasi-MCMC methods.

FIGURE H.4. Actual and Estimated Disengagement (assuming a Logistic Distribution)



Notes: The figure plots actual disengagement a and expected disengagement $\sum_d \tilde{P}(d)(H(c(d, a)))$ at the estimated θ^* , κ^* , and σ^* in blue. Here, we assume a logistic distribution for H during the estimation so that $H(c(d, a)) = 1/(1 + e^{((-\theta^* d * a) + \kappa^*)/\sigma^*})$.